

The GLP-1 Companion

A Woman's Guide to Weight-Loss
Medication, Eating for Muscle, and
Keeping It Off for Good

Hannah Wells

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For every woman who has been told to just eat less
and move more, as if it were that simple —
this is the book I wish someone had handed you
the day your doctor first mentioned the injection.

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Foreword: The Conversation You've Been Waiting For

YOU'VE BEEN THINKING ABOUT THIS FOR A WHILE. MAYBE YOUR DOCTOR MENTIONED IT. MAYBE A FRIEND LOST FORTY POUNDS AND LOOKS LIKE A DIFFERENT PERSON. MAYBE YOU'VE READ THE HEADLINES — THE SHORTAGE SCARES, THE INSURANCE BATTLES, THE MIRACLE STORIES, THE HORROR STORIES. AND SOMEWHERE IN THE MIDDLE OF ALL THAT NOISE, YOU'RE TRYING TO FIGURE OUT ONE THING: IS THIS FOR ME?

This book is the conversation I wish someone had with me before I took my first injection.

I remember sitting in my car in the pharmacy parking lot, holding a small white box that cost more than my weekly grocery budget, feeling equal parts hopeful and terrified. Nobody had given me a manual. The doctor had spent eleven minutes with me. The internet had given me a hundred conflicting opinions. I was about to change my body, my appetite, and possibly my life — and I had no idea what I was doing.

That was two years ago. Since then I've lost the weight, kept it off, learned more about protein than I ever wanted to know, thrown up in a Target parking lot (once), and rebuilt my relationship with food from the ground up. I've

also read the research, talked to dozens of women on these medications, and collected every practical trick that actually works.

Here's what I know now: the medication is the easy part. The shot takes thirty seconds. The hard part — and the part that determines whether you keep the weight off — is everything around it. What you eat. How you train. How you handle the nausea. How you manage the scale when it stalls. How you think about food when the food noise finally goes quiet.

This book is not medical advice. I'm not a doctor, and I won't pretend to be one. It's a companion — the practical, honest, from-the-trenches guide to getting the most out of these medications while protecting your health, your muscle, and your sanity.

Let's get started.

Toolkit Summary

- GLP-1 medications are powerful tools — but they're only half the equation.
- The shot is easy; the lifestyle around it determines your long-term success.
- This book covers the practical side: food, muscle, side effects, and maintenance.
- You are not alone, and you are not the first person to feel overwhelmed by this decision.

Chapter 1: The GLP-1 Revolution: What These Medications Actually Do

LET'S START WITH THE MOST IMPORTANT THING I LEARNED, AND THE THING ALMOST NOBODY TOLD ME BEFORE I STARTED: THE MEDICATION IS NOT A DIET. IT'S NOT A WEIGHT-LOSS PROGRAM. IT'S NOT A MAGIC INJECTION THAT MELTS FAT WHILE YOU KEEP LIVING EXACTLY THE WAY YOU'VE BEEN LIVING. IT'S A BIOLOGICAL RESET BUTTON — A WAY OF TURNING DOWN A SIGNAL IN YOUR BODY THAT HAS BEEN SHOUTING AT YOU FOR YEARS, POSSIBLY DECADES. AND WHEN THAT SIGNAL FINALLY GOES QUIET, EVERYTHING ELSE BECOMES POSSIBLE.

This chapter is the foundation. If you understand what these medications actually do — not what the headlines say, not what your cousin's hairdresser's sister said, but what the science says — then every decision you make from here gets easier. You'll know why you feel what you feel in month one. You'll know why protein matters so much. You'll know why the weight can come back if you stop. And you'll know, most of all, that you are not weak for needing this. You are not broken. You have been fighting a biological system with willpower alone, and that was never a fair fight.

What GLP-1 medications actually are

Here's the part that surprised me most: these drugs aren't foreign chemicals invented in a lab to make us skinny. They're based on a hormone your body already makes. GLP-1 stands for glucagon-like peptide-1, and it's a naturally occurring hormone your gut releases when you eat. Its job, in normal amounts, is to tell your brain you've eaten, to help your pancreas release insulin, and to slow down digestion so your body has time to actually use the food.

In a healthy system, that works beautifully. You eat, the hormone fires, you feel satisfied, you stop. Done.

For a lot of us, that system is quiet or broken. The signal is weak, or it gets overridden by a thousand other signals — stress, habit, the dopamine hit of food, the constant advertising, the emotional history we all carry around with food. So we eat past the point of fullness, we think about food when we're not hungry, we graze, we snack, we finish the bag. Not because we're weak — because the "stop eating" message never arrived, or arrived too quietly to be heard.

GLP-1 medications are essentially a supercharged version of that hormone. They're given as a weekly injection, and they keep that "I'm full" signal running at a level your body never produces on its own. The result is that the volume gets turned down on hunger itself.

That's it. That's the revolution. It's not about burning more calories or suppressing your appetite with stimulants. It's about restoring a signal your body was supposed to have in the first place.

How they work: the three dials

If you want to understand why these medications feel so different from every diet you've ever tried, you need to understand the three things they actually do. Think of them as three dials in your body, and the medication turns all three at once.

Dial one: appetite signaling. This is the gut-brain connection. When you take a GLP-1 medication, it travels to receptors in your brain — specifically the areas that regulate appetite and reward — and tells them, in effect, that you've eaten enough. You feel full sooner. You feel full longer. And crucially, you feel satisfied sooner. The signal isn't "you're not allowed to eat." The signal is "you genuinely don't need to." That's a completely different experience from white-knuckling through a diet. On a diet, you're hungry all the time and you're fighting it. On a GLP-1, most women describe a strange, calm disinterest in food. Not disgust. Not denial. Just... quiet.

Dial two: gastric emptying. This one is a double-edged sword, and it's responsible for both the fullness and the nausea. These medications slow down how quickly food moves from your stomach into your small intestine. Food sits in your stomach longer, which means

you feel full for hours after a small meal. It's why a normal-size plate suddenly feels like a banquet. But it's also why greasy food, big meals, and eating too fast can make you miserable — the food has nowhere to go, and your stomach is not happy about it. This is the dial that requires you to learn to eat small, and it's the dial we'll spend a whole chapter on later.

Dial three: food noise reduction. This is the one that changes everything, and it deserves its own section.

What "food noise" is, and why its silence changes everything

I want you to think about the last time you were trying to work, or drive, or watch a movie, and your brain was running a low-level commentary about food. What am I going to eat later? Should I stop for something on the way home? There's leftover pizza in the fridge. Maybe just a bite. What time is dinner? I could order something. No, I should cook. But the takeout place is right there...

That constant background chatter — the voice that plans, negotiates, fixates, and obsesses over food even when you're not hungry — that's food noise. For women with a certain kind of relationship with food, it never stops. It's there at 10 a.m. and 2 p.m. and 11 p.m. It's exhausting. And here's the cruelest part: every diet you've ever been on probably made it louder, because restriction

is catnip to a food-obsessed brain. You tell yourself you can't have the cake, and suddenly the cake is all you can think about.

The first few weeks on a GLP-1, something strange happens. The voice just... stops. Not because you're fighting it, but because the medication has turned down the signal it was broadcasting from. Women describe this moment in almost identical words, over and over: "I realized I hadn't thought about food all day." "I walked past the snack aisle and felt nothing." "I forgot to eat lunch — me, who has never forgotten a meal in her life."

That silence is not willpower. You didn't get stronger. The signal got quieter. And when the signal is quiet, you finally get to make food decisions with your actual preferences instead of your compulsions. You get to choose the salad because you want it, not because you're being punished. You get to eat half and stop, not because you're being "good," but because you're full.

I can't overstate how life-changing that is. But I also need to be honest with you, right here at the beginning: that silence is a gift, not a guarantee. The food noise is still in there. It comes back when you stop the medication. That's why this book is as much about the months and years after the weight comes off as it is about the weight coming off. But more on that later.

The history: from diabetes drug to blockbuster

Here's a timeline that helps explain why you're suddenly hearing about these drugs everywhere.

The story starts in the 2000s with the first GLP-1 medication, exenatide, approved to treat type 2 diabetes. Diabetes doctors noticed something interesting almost immediately: their patients were losing weight. Not a little — a lot, especially compared to other diabetes drugs, many of which caused weight gain. But weight loss wasn't the point yet. These were diabetes drugs, full stop.

Over the next decade and a half, the drugs got better. Liraglutide, then semaglutide, then tirzepatide — each generation more effective than the last. And as the data piled up, the medical establishment started to take the weight-loss effect seriously. In 2014, liraglutide was approved for weight management. In 2021, semaglutide — under the brand name Wegovy — was approved for weight loss, after clinical trials showed participants losing an average of around 15 percent of their body weight. Then in late 2023, tirzepatide, under the brand name Zepbound, hit the market with trials showing average losses over 20 percent.

Those numbers changed everything. Fifteen to twenty percent of your body weight is not a diet result. That's a bariatric-surgery-adjacent result, achieved with a weekly shot. Suddenly these medications weren't a niche diabetes treatment — they were the biggest thing to

happen to weight management in modern medical history. Demand exploded. Shortages followed. Celebrities talked about them. Your neighbor started taking one. The FDA approved Wegovy for reducing cardiovascular risk in 2024, which expanded insurance coverage for a lot of people.

And that's where we are now: at the beginning of a genuine revolution in how we treat obesity — as a chronic biological condition instead of a character flaw.

The lineup: which medication is which

If you've been reading headlines, the brand names probably blur together. Let me sort them out.

Semaglutide is the molecule in two famous brands. As **Ozempic**, it's approved for type 2 diabetes. As **Wegovy**, it's approved for weight management (and cardiovascular risk reduction). Same active ingredient, different names, different approvals, different insurance coverage rules. This is a key distinction: your insurance might cover Ozempic if you have diabetes, and deny Wegovy if you don't, even though it's the same drug.

Tirzepatide is the newer molecule — a "dual" medication that targets GLP-1 plus a second hormone pathway (GIP). As **Mounjaro**, it's approved for diabetes. As **Zepbound**, it's approved for weight loss. It's the one with the eye-popping trial numbers — average losses

north of 20 percent — though real-world results vary, and "average" means some people do better and some do worse.

Liraglutide is the older, shorter-acting option (a daily injection rather than weekly). It's less potent and less convenient, but it's been around longer and is sometimes covered where the newer drugs aren't. It's a perfectly reasonable place to start if that's what your situation allows.

Compounded versions are copies of semaglutide and tirzepatide made by compounding pharmacies, which became legal to produce during the drug shortages. They're cheaper, often dramatically so, and they're a complicated topic with real risks and real benefits — we'll dig into the full picture in the next chapter.

One more thing you should know before we move on: I'm not going to give you dosages in this book, and neither should anyone who isn't your prescriber. These medications follow a titration schedule — you start low and increase gradually over weeks, per your prescriber's instructions, to let your body adjust. The specific numbers are between you and your doctor. What I can tell you is what the experience feels like, and that's coming in the chapters ahead.

Realistic expectations: what the numbers actually look like

Let's talk numbers, because the internet is full of both miracle stories and horror stories, and neither is the full truth.

In clinical trials, the averages were roughly 15 percent of body weight for semaglutide and around 20 percent for tirzepatide over about 16 to 18 months. In the real world, results tend to be a bit more modest — real people aren't in tightly controlled trials, and real people sometimes stop early or don't follow the eating guidance. A realistic, good outcome for many women is somewhere in the 10 to 20 percent range. For a 220-pound woman, that's 22 to 44 pounds. For a 190-pound woman, 19 to 38.

Notice I said "over 16 to 18 months." This is a marathon, not a sprint. The weight typically comes off fastest in the first few months, then slows down, then plateaus — and the plateau is normal. It's not failure. It's what happens when your body adjusts to its new set point. The women who do best are the ones who treat the plateau as part of the process instead of a reason to quit.

Also important: not everyone loses a dramatic amount. Some people are "non-responders" — the medication simply doesn't move the needle much for them, and that's nobody's fault. And some people lose weight quickly and then hit a wall. Your results will be yours. Comparing your curve to someone else's is a guaranteed way to make yourself miserable.

And here's the expectation that matters most of all: **these medications work while you're on them.** The appetite suppression, the food noise silence, the weight loss — they're all tied to the medication. When you stop, the signal comes back, and without a plan, the weight tends to come back too. This is so common it has a name — the "GLP-1 cliff" — and it's the reason this book exists. The women who keep the weight off are the ones who use the quiet years to build the habits, the muscle, and the food skills that will carry them after the medication is gone.

Why this is different from every diet that came before

Every diet you've tried asked you to override your biology with willpower. Eat less. Resist the craving. White-knuckle through the hunger. And when you "failed" — when the hunger won, as hunger always eventually does — you concluded you were weak. You were not weak. You were running a marathon with your hands tied behind your back, and then blaming yourself for not winning.

This is different. This is not a diet. This is a medication that changes the biological conditions under which you're trying to make changes. The hunger is quieter. The fullness comes sooner. The food noise stops. And for the first time, your effort — the part of this that genuinely is

on you — can go toward things that actually work: eating enough protein, building muscle, learning what real fullness feels like, rebuilding your relationship with food.

Here's my honest summary of the revolution: these drugs don't do the work for you. They make the work possible. And if you've spent years believing the work was impossible because you were broken — this is the chapter where I tell you, as gently and as firmly as I can: you were never broken. The system was rigged. Now you have a way to un-rig it.

Toolkit Summary

- GLP-1 medications are based on a hormone your body already makes — they amplify a "you're full" signal, not a foreign chemical.
- They work through three dials: appetite signaling in the brain, slower gastric emptying, and the reduction of "food noise."
- Food noise is the constant background chatter about food; its silence is often the most life-changing effect of these medications.
- The lineup: semaglutide (Ozempic for diabetes, Wegovy for weight loss), tirzepatide (Mounjaro for diabetes, Zepbound for weight loss), liraglutide (older, daily), and compounded versions made during shortages.

- Realistic results: roughly 10–20 percent of body weight over a year or more — with a normal plateau along the way. Individual results vary widely.
- These medications work while you're on them; the "GLP-1 cliff" (weight regain after stopping) is real and is why the habits matter.
- This is not a diet. It's a tool that makes the work possible — the work of protein, muscle, and a rebuilt relationship with food.

Chapter 2: Is This Right for You? The Honest Decision

HERE'S THE THING NOBODY TELLS YOU ABOUT THE DECISION TO START A GLP-1 MEDICATION: IT'S TWO DECISIONS, NOT ONE. THERE'S THE MEDICAL DECISION — DO THE BENEFITS OUTWEIGH THE RISKS FOR YOUR BODY, YOUR HISTORY, YOUR SITUATION? AND THERE'S THE EMOTIONAL DECISION — ARE YOU READY FOR WHAT THIS IS GOING TO CHANGE, AND ARE YOU DOING IT FOR THE RIGHT REASONS?

Most of the conversation around these drugs only covers the first one. The doctor's office covers the first one. The headlines cover the first one. But in my experience, it's the second decision — the emotional one — that determines whether this becomes a tool you use well or a chapter of your life you regret. So let's do both, honestly, the way I wish someone had walked me through it.

Who these medications are actually for

Let's start with the official criteria, because it's useful to know what the FDA actually approved these drugs for. For the weight-loss versions, the approval is generally for adults with a body mass index (BMI) of 30 or higher, or a

BMI of 27 or higher with at least one weight-related condition — things like high blood pressure, type 2 diabetes, high cholesterol, or sleep apnea.

If you're reading those numbers and doing the math in your head — if your BMI is 26, or 24, and you're wondering whether you "qualify" — I want to pause here. The BMI cutoff is a regulatory line, not a moral one. It exists to define who the clinical trials studied, not to define who deserves help. Doctors vary in how strictly they apply it. Some will prescribe to a woman with a BMI of 26 if her blood pressure is climbing and her joints hurt. Some won't. That's the reality of a system that uses a blunt tool like BMI.

What I can tell you is that the spirit of the criteria matters more than the letter. These medications were studied in people whose weight was affecting their health — physically, medically, or both. The question that matters isn't "does my number qualify?" It's "is my weight causing me real problems, and have I genuinely tried the other tools?" If the answer to both is yes, you have a legitimate case. If you're a woman with a BMI of 23 who just wants to drop ten pounds for a wedding, this is not your tool — and any honest prescriber will tell you the same.

The honest self-assessment

Before you book the appointment, I want you to sit with a few questions. Not the ones your doctor will ask — the ones only you can answer.

What have you actually tried? Be honest with yourself about the full history. The diets, the programs, the trainers, the apps, the "this time will be different" moments. If you've spent years of your life cycling through restriction and regain, that's not a character flaw — it's evidence that your biology is fighting you, which is exactly the situation these drugs were designed for.

How is your weight affecting your life, right now, today? Not in the abstract — concretely. Are your knees hurting on stairs? Is your blood pressure creeping up? Is your energy so low that you're canceling plans? Are you avoiding photos, or flights, or chairs with arms? Write these down. These are your reasons, and they're valid.

What's your relationship with food, really? Do you think about food constantly? Do you eat when you're not hungry? Do you hide what you eat? Do you feel out of control around certain foods? If any of this is familiar, you're not alone — and it's relevant, because the food noise piece is where these drugs do some of their most profound work.

What's your plan for the other side? Here's the question almost nobody asks before starting: what happens when the weight is off and the medication eventually stops? Because it usually eventually stops —

for cost, for insurance, for pregnancy, or just because you and your doctor decide it's time. The women who succeed long-term don't start the medication with a plan for after. They start with a plan for after. We're going to spend a lot of this book building that plan, so the fact that you're reading it already puts you ahead.

The medical history conversation

This is the part where I have to be careful, because I am not a doctor and this book is not medical advice. But there are a few things every woman should know before her appointment, so she can ask the right questions and advocate for herself.

Thyroid. There's a specific and rare type of thyroid cancer — medullary thyroid carcinoma, or MTC — that these medications carry a warning about. If you have a personal or family history of MTC, or a condition called MEN2 (multiple endocrine neoplasia syndrome type 2), these drugs are generally considered off-limits. Your doctor should ask about this. If they don't, bring it up yourself.

Pancreas. These medications are associated with an increased risk of pancreatitis — inflammation of the pancreas. If you've had pancreatitis before, or if you have a history of gallstones or very high triglycerides (which can trigger it), your doctor will want to weigh that carefully.

Gallbladder. Rapid weight loss of any kind — diet, surgery, or medication — increases the risk of gallstones. If you have a history of gallbladder problems, or if you still have your gallbladder and it's been grumpy, say so. This doesn't necessarily disqualify you, but it changes the conversation and the monitoring.

Everything else. Pregnancy is a hard no — these drugs aren't studied in pregnancy, and you should be off them well before trying to conceive. If you take oral birth control, there's an important wrinkle: slower gastric emptying can affect how well oral medications are absorbed, so you and your doctor should talk about that. Same for any other daily oral medication. And if you have a history of depression or an eating disorder, have an honest conversation about it — for some women these drugs interact with that history in complicated ways, and for others they're genuinely healing. Your doctor needs the full picture to help you.

Here's the meta-point: the medical history conversation is a conversation, not a verdict. A history of gallstones doesn't automatically mean no. A BMI of 28 with sleep apnea might absolutely mean yes. The goal of this checklist is not to scare you — it's to make sure you walk into that appointment informed enough to ask the right questions instead of nodding along.

The cost and insurance reality

Okay, deep breath. This is the part that makes everyone's eyes glaze over, and it's also the part that determines whether this whole thing is even feasible for you. Let's make it as clear as I can.

Without insurance, these drugs are expensive. List prices are typically in the range of \$1,000 to \$1,300 or more per month — that's per month, not per course of treatment. For most people, that's not sustainable out of pocket, which is why the insurance piece matters so much.

With insurance, coverage varies wildly. Some plans cover weight-loss medications, some don't. If your plan does cover them, you'll often face a **prior authorization** — a process where your doctor submits paperwork to the insurance company proving you meet their criteria before they'll pay. Expect this to take time, possibly weeks, and expect to chase it. Expect possible denials and appeals. Expect some plans to require **step therapy** — meaning you have to try and fail a cheaper medication first. This process is exhausting, and it's not your fault, and it's worth pushing through because the difference between paying \$25 a month and \$1,200 a month is life-changing.

Coupon programs exist but have limits. The manufacturers offer savings cards that can bring the price down significantly — but they typically only work if you have commercial insurance, and they often exclude people on Medicare or Medicaid. Also important:

Medicare does not cover weight-loss medications as of this writing, full stop, which leaves a lot of women over 65 in a brutal position. This landscape changes constantly, so treat anything you read — including this book — as a starting point, and check current rules with your plan and the manufacturer's website.

The compounding question

If you've been looking into this at all, you've probably seen ads for "compounded semaglutide" at a fraction of the price. Here's what that is and what you need to understand.

Compounding pharmacies make personalized versions of medications — it's a legitimate part of the medical system, used for things like custom dosages or allergy-friendly formulations. Normally, they can't copy a brand-name drug like semaglutide. But during the drug shortages, the FDA allows compounding of the drugs in shortage, which is how compounded semaglutide and tirzepatide became widely available. The shortages are easing in some places and not others, and the FDA situation around compounded versions has been shifting — with the agency and the manufacturers pushing to wind down compounding now that supply is improving, and the compounding industry pushing back. It's an ongoing legal and regulatory battle, and the rules can change on short notice.

Here's what you need to know practically. Compounded versions are cheaper — often dramatically — which makes them the only option for many women. That's a real benefit. But they are not FDA-approved in the way the brand-name drugs are. There's no guarantee of identical ingredients or dosing, and there have been reported cases of compounding pharmacies getting the strength wrong, using unapproved "salt" versions of the drugs, or selling products of questionable sterility. The gray market — buying "research peptides" online from unregulated sellers — is genuinely dangerous and should be off the table entirely.

If you're considering compounded medication, do it through a licensed, accredited compounding pharmacy that requires a real prescription from a real doctor, and have an honest conversation with your prescriber about what you're getting. This is one of those areas where "cheaper" and "safe" are not automatically the same thing.

The alternatives worth knowing about

Before you commit to this path, know what the other paths look like — because the right answer for you might not be a GLP-1 at all, and you deserve to choose with your eyes open.

Lifestyle-first. Structured, medical-grade lifestyle programs — the kind with real coaching, meal structure, and accountability — genuinely work for some women,

especially combined with the other tools below. The honest caveat is that for women with significant weight to lose and a long history of regain, lifestyle alone often isn't enough. But it's a legitimate first stop, and many prescribers want to see it tried before they reach for a prescription.

Other medications. There are older weight-loss drugs — phentermine, naltrexone-bupropion, topiramate-based combinations, orlistat — that are cheaper, sometimes easier to get covered, and helpful for some women. They generally produce more modest results than the GLP-1s, and some come with side effects of their own (jitteriness, sleep problems, and the like). They're not the shiny new thing, but they're real tools.

Bariatric surgery. For women with higher BMIs or significant weight-related health conditions, surgery remains a highly effective option — often more durable than medication, because the anatomy change is permanent. It's a bigger commitment with real risks, and insurance coverage has its own gauntlet, but it deserves to be on the table in the honest conversation.

None of these are "the easy way out," by the way. There is no easy way out. There's only the tool that fits your body, your finances, and your life.

The emotional decision: for you, or for other people?

And now we get to the question I promised we'd ask. Sit with it for a minute.

Are you doing this for you — or for other people?

Because here's what I see over and over in the women I've talked to. The ones who do well are doing it for their knees, their energy, their sleep, their blood pressure, their ability to run after their kids, their future. The ones who struggle — the ones who feel shame about the injection, who hide it from their friends, who panic when the scale stalls, who quit at the first side effect — are often doing it for an imagined audience. The ex who will regret it. The high school classmates at the reunion. The voice of a mother who commented on their body. The version of themselves they think they're supposed to be.

Here's the truth I need you to hear, and I'm going to say it as directly as I can: you don't owe anyone a smaller body. Not your partner, not your family, not the internet, not the ghost of your thinner self. If you start this medication because you're trying to become acceptable to other people, you will never be small enough — because that's not a size, that's a wound, and no injection can fix it.

Doing this for yourself looks different. It means wanting more energy, not just a smaller dress. It means wanting to protect your health so you can be present for the next thirty years. It means wanting to stop the food noise

because it's exhausting, not because you're ashamed of what you eat. It means treating the medication as a tool you're choosing — not a secret you're keeping.

And here's the beautiful, strange thing I discovered: when the food noise goes quiet and you're not constantly at war with your own appetite, you find out what you actually want. Not what you were told to want. What you want. That's when the real work — and the real payoff — begins.

So before you book the appointment, ask yourself the question. Not "am I fat enough to deserve this?" Not "will people judge me?" The question is: **do I want this for me?** Because if the answer is yes, then we can build everything else on top of that. And if the answer is no — if this is for someone else's approval — then the kindest thing you can do is wait until the answer changes.

Toolkit Summary

- These medications are approved for BMI 30+, or BMI 27+ with a weight-related condition — but the real question is whether your weight is affecting your health and you've genuinely tried other tools.
- Do the honest self-assessment: what you've tried, how weight affects your life today, your relationship with food, and your plan for after the medication.

- Medical history to raise with your doctor: thyroid (MTC/MEN2), pancreas (pancreatitis), gallbladder, pregnancy plans, oral medications including birth control, and mental health history.
- Cost reality: roughly \$1,000+ per month out of pocket; insurance coverage varies, prior authorizations are common, and manufacturer coupons only help with commercial insurance.
- Compounded versions are cheaper but not FDA-approved like the brands; if you go that route, use a licensed pharmacy with a real prescription — never the gray market.
- Alternatives exist: structured lifestyle programs, older medications, and bariatric surgery — none of them are "the easy way out."
- The emotional decision matters most: choose this for yourself, not for other people's approval — no size will ever be small enough to fix someone else's voice in your head.

Chapter 3: Before You Start: Blood Work, Expectations, and the Food Noise Problem

THE WEEKS BEFORE YOUR FIRST INJECTION ARE A STRANGE IN-BETWEEN TIME. YOU'VE MADE THE DECISION — OR YOU'RE CLOSE TO IT — AND NOW YOU'RE WAITING. WAITING FOR THE PRIOR AUTHORIZATION. WAITING FOR THE PHARMACY TO HAVE IT IN STOCK. WAITING FOR THE DAY YOU PICK UP THAT LITTLE WHITE BOX AND EVERYTHING CHANGES.

Here's the thing about that waiting period: it's actually the most useful time in the whole process. It's the only time you'll have to prepare — your body, your kitchen, your family, your expectations — while you still have a full appetite and a clear head. The women who do best on these medications are almost always the ones who did their homework in the weeks before the first shot. This chapter is that homework.

The blood work: what to get checked before you start

I'm not a doctor, and this is not medical advice — but every responsible prescriber will want a baseline before you start, and it helps to know what you're looking at when the lab results come in.

The full blood panel. A complete blood count (CBC) and a comprehensive metabolic panel give the doctor a picture of your overall health — your blood cells, your liver enzymes, your kidney function. These are the boring-but-essential basics that catch problems early.

A1C. This is your average blood sugar over the past few months, and it matters even if you've never had a diabetes diagnosis. Many women who think of themselves as "not diabetic" are surprised to find their A1C creeping into the prediabetes range. Your starting A1C also tells you something important about how your body processes food — and it gives you a second metric to watch, beyond the scale, as you go.

Thyroid. A TSH (thyroid-stimulating hormone) test checks that your thyroid is functioning normally. This matters twice over: thyroid problems can cause weight gain or make weight loss harder, and there's the thyroid cancer consideration we talked about earlier. Knowing your baseline is good medicine on both counts.

Vitamin and nutrient levels. This one is easy to overlook, and it's a favorite of mine, because it quietly prevents so much misery. When your appetite drops and you start eating much less, your nutrient intake drops too — and if you were already low on things like vitamin D, iron, or B12, the deficiency can hit you hard a few months in (fatigue, hair changes, low mood). Getting your levels checked now means you can top up before you start. Trust me, you'd rather fix a vitamin D deficiency in week zero than discover it in month three.

Gallbladder and liver history. If you've ever had gallstones, gallbladder attacks, or gallbladder surgery — or if your liver enzymes have ever been elevated — make sure your doctor knows, and that it's on your chart. Rapid weight loss stresses the gallbladder, and it's better to know your starting point than to be surprised in month four.

A quick note on timing: get this done a few weeks before your start date, not the week of. That gives your doctor time to review results, flag anything, and adjust the plan — and it gives you time to fix any deficiencies before they become problems.

What to ask your doctor

You're going to get eleven minutes with your prescriber if you're lucky, so use them well. Write these down and take the list with you. The questions that actually matter:

- "What's the titration schedule you use, and what should I expect at each dose increase?"
- "What side effects are normal, and which ones should make me call you? What about which ones should send me to the ER?"
- "How should I adjust if I'm having a rough week — do we slow down the titration, or push through?"
- "I take [your medications, including birth control and any daily pills]. Will slower digestion affect any of them?"
- "What should I do if I miss a dose?"
- "How do we decide when it's time to stop, and what does stopping look like?"
- "Are there any supplements or foods I should avoid in the first few weeks?"

And the meta-question behind all of them: what's your philosophy here? Some prescribers are aggressive — push through side effects, hit the top dose fast. Some are conservative — slow titration, low doses, "let's see how you do." Neither is wrong, but you want to know which one you're working with, because it will shape your whole experience.

Telling your family (or not telling them)

Here's a decision that's more emotional than practical, and I want to give you permission to make it however you need to.

You don't have to tell anyone. I know a woman who told her husband, her mother, and her best friend, and I know women who have taken these shots for over a year and told exactly one person. There is no obligation to broadcast your medical decisions, and there's a real argument for privacy: other people's opinions — the "isn't that cheating?" comments, the "you should just eat less" lectures, the worry, the judgment — can land on you at the exact moment you need support, and land hard.

But here's the other side: the practical reality is that this medication will be visible. The injections, the food changes, the nausea days, the weight loss itself — people will notice, and the women who thrive are usually the ones who have at least one person who knows the whole story. Not everyone. One person. A partner, a sister, a friend who gets it. Someone who can hold the bag while you throw up in the Target parking lot and not make it weird.

If you do tell people, here's a script that works: "I'm starting a medication for my weight, under my doctor's care. It's a serious medical decision I've thought about for a long time. I'm telling you because I trust you, and I need you to be on my team — which means no commentary on my food or my body, just support." You'd be surprised how

well "no commentary on my food or my body" works when you say it out loud, in advance, calmly. It sets a boundary before the boundary is needed.

And if you get the "isn't that the easy way out?" question — you don't have to defend yourself. You're not in a debate. "My doctor and I decided it's right for me" is a complete sentence.

Setting realistic expectations: the shape of the journey

Let me draw you the actual weight-loss curve, because I promise it doesn't look like the one in your head.

The first month is mostly an adjustment period. Some women lose weight immediately — a few pounds of water weight, usually — and some lose almost nothing. Both are normal. Your body is getting used to the medication, you're learning how to eat on it, and the dose is still low. If you judge the whole journey by month one, you'll misjudge it.

Months two through six are where most of the visible change happens. The dose has built up, the appetite suppression is real, and the pounds come off at a steady clip — usually faster early and slowing as you go. This is the fun part, and also the part where you're most tempted to under-eat, which we'll deal with in the protein chapters because it matters enormously.

Months six through twelve and beyond are the consolidation phase. The loss slows, then stalls, then — for most people — settles at a plateau. The plateau is not a failure. It's your body finding its new set point. The women who quit at the plateau are the ones who regain; the women who accept it and keep building habits are the ones who keep the weight off.

And the honest truth about the whole arc: this is a year-long project, minimum. If you're looking for a six-week transformation, this isn't it — and you don't want it to be, because the six-week transformations are exactly the ones that don't last.

Preparing your kitchen

Here's a practical truth about starting these medications: your appetite is going to drop, and your tolerance for certain foods is going to change, possibly overnight. The food you loved last week may make you nauseous next week. So the week before you start, you want a kitchen that works with your new body, not against it.

What to clear out: the foods that will become landmines. If it's greasy, heavily fried, or rich, it's probably going to make you miserable in month one — and more importantly, you don't need it around when you're adjusting. Donate the unopened stuff. Throw away the half-eaten bags. Get the trigger foods out of the house

now, while you still have the willpower to do it, because in three weeks you won't need willpower — but you also won't want a cabinet full of regret.

What to stock up on: the gentle, boring, reliable foods. Plain crackers, white rice, bananas, applesauce, broth, plain chicken, eggs, ginger tea, electrolyte drinks, protein shakes. You don't need a gourmet pantry — you need a survival pantry, and we'll build the full list in the nausea chapter. Buy the basics now so you're not standing in a grocery store at 7 p.m. on day three feeling queasy and overwhelmed.

The food noise awareness exercise

This is my favorite exercise in the whole book, and it's only possible before you start, so don't skip it. Here's the assignment:

For the week before your first injection, pay attention to the food noise — and write it down.

Every time you think about food when you're not hungry, notice it. Not judgmentally — just notice. What time is it? What triggered it — a commercial, a smell, stress, boredom, a memory? What does the voice actually say? "You deserve a treat." "You can start Monday." "One bite won't hurt." "What if they run out of the good kind?" And what does it feel like in your body — a tightness, a craving, a restlessness?

Keep a running list for seven days. That's it. That's the whole exercise.

Here's why it matters. In a few weeks, that voice is going to go quiet — and when it does, you'll want to remember what it sounded like. Not because you're nostalgic for it, but because understanding it is what keeps it from coming back with a vengeance when the medication stops. The women who regain weight after stopping are often the ones who never understood their food noise in the first place — they just enjoyed the silence and assumed it was permanent. The women who keep the weight off are the ones who studied the noise while they had the chance, and built other ways of responding to its triggers — the stress that made them reach for food, the boredom, the reward-seeking — before the silence ended.

Also, practical bonus: when you write the food noise down, you realize how much of your day it was consuming. That awareness alone is worth the price of the book.

Scheduling your first dose: pick the right day

This sounds trivial. It is not trivial. The day and time of your first injection can genuinely shape your first month, so let's think it through.

Pick a day when you can be gentle with yourself.

Not the day before a big presentation. Not the day of your daughter's birthday party. Not the day you're flying cross-country. The first 24 to 48 hours after a dose are when any side effects will announce themselves, and you want to be home, hydrated, and low-stress when they do. For most women, that means a Friday evening or a Saturday morning — which conveniently also means the worst of any side effects land on the weekend.

Pick a time of day that works with your body. Some women prefer morning injections (the nausea is less noticeable during the day, and it doesn't interfere with sleep). Some prefer evening, right before bed — they sleep through the worst of any queasiness. There's no right answer; there's only your answer. If you're prone to nausea, evening is often the gentler start.

Pick a day of the week you'll remember. The same day every week, ideally tied to an existing anchor — Sunday morning coffee, Friday after work. Put it in your phone with a reminder. Missing doses is one of the most common reasons people stall, and it's almost always just a scheduling problem.

And plan the week around it, at least at first. Keep the day after your injection low-key. Have the easy foods on hand. Have the ginger tea ready. Don't plan a restaurant dinner for day two. After a month or two, you'll barely notice injection day — but in the beginning, treat it like what it is: a new thing your body is learning.

One last thing, and it's the most important expectation of all: **the first dose is not the finish line — it's the starting line.** You may feel nothing at all in week one. You may feel a little off. Either way, the real changes build over weeks as your dose rises. Don't judge the whole journey by day three. Judge it by the arc — and give the arc time.

Toolkit Summary

- Get baseline blood work before you start: full panel, A1C, thyroid (TSH), vitamin levels (D, B12, iron), and gallbladder/liver history on the record.
- Bring a written list of questions to your prescriber: titration plan, side-effect triage, interactions with your other medications, and how stopping will work.
- Telling family is optional — one trusted person is enough; use a script that sets boundaries ("no commentary on my food or my body").
- Know the shape of the journey: month one is adjustment, months two through six bring most of the change, and the plateau later is normal, not failure.
- Clear the trigger foods out of your kitchen now, and stock the gentle basics before you start.

- Do the food noise awareness exercise for one week — write down when the voice talks, what triggers it, and what it says. You'll need that map later.
- Schedule your first dose for a low-stress day, a time that suits your body, and a weekday you'll remember — and plan to be gentle with yourself for the 48 hours after.

Chapter 4: Your First Month: Side Effects, Titration, and Getting Through Week One

LET ME TELL YOU WHAT THE FIRST MONTH IS ACTUALLY LIKE, BECAUSE THE INTERNET WILL TELL YOU TWO VERSIONS AND BOTH OF THEM ARE WRONG. THE FIRST VERSION: YOU TAKE THE SHOT, THE WEIGHT MELTS OFF, YOU FLOAT THROUGH LIFE IN A STATE OF SERENE APPETITE-FREE BLISS. THE SECOND VERSION: YOU TAKE THE SHOT AND IMMEDIATELY SPEND A MONTH HUNCHED OVER A TOILET, REGRETTING EVERYTHING.

The truth is somewhere in between, and it's much more boring than either: the first month is mostly an adjustment period. For most women it's a little weird, occasionally uncomfortable, and — this is the part nobody warns you about — sometimes completely uneventful. All of those are normal. Let me walk you through it week by week, so you know what's coming and what's actually worth worrying about.

What to expect, week by week

Week one. You take your first injection, at the low starting dose, and then... probably nothing dramatic. Some women feel a mild queasiness in the first 24 to 48 hours. Some feel slightly fatigued. Some feel a subtle change in appetite — "I noticed I didn't finish my plate" — and some feel nothing at all. Here's the crucial thing: **an uneventful week one is completely normal.** The starting dose is deliberately low, and the medication builds up in your system over weeks. It's not working less for you because you don't feel it yet. It's doing exactly what it's supposed to do: easing your body into a change it's never experienced before.

Weeks two and three. This is usually where the effects become noticeable. The appetite suppression kicks in — you'll have moments of "oh, I actually forgot to eat" — and the food noise starts to fade. This is also the window where side effects, if you're going to have them, tend to announce themselves, because the medication is now present in your system at a meaningful level. If you're nauseous or tired, this is when it typically shows up. It's also when many women make the mistake I want to warn you about right now: the appetite drops, the food noise goes quiet, and you get excited, so you eat almost nothing. That's the fastest route to feeling terrible — weak, nauseous, headachy, and miserable. We'll get deep into the protein fix for this in the next chapter, but for now, file it away: **under-eating is not a strategy, it's a mistake.**

Week four and beyond. By now, you're learning your body's new rhythms. You know which foods work and which don't. You've survived your first few injection cycles. You may have lost a few pounds, or you may not have — and both are fine. Your prescriber will likely be talking about increasing your dose around now, which brings us to the most important concept in the first month: titration.

Titration: why starting low matters so much

"Titration" is the medical word for ramping up. You don't start on the full dose — you start low and increase gradually, on a schedule your prescriber sets, over a period of weeks or months. This is not arbitrary. It's the single most important factor in how well you tolerate the medication.

Here's why: your body has never seen these elevated GLP-1 levels before. If you flooded it with the full dose on day one, you'd be violently ill — the nausea, vomiting, and misery that make the horror stories. Starting low gives your system time to adapt, building up tolerance week by week. The drug companies designed the titration schedule specifically to minimize side effects, and your prescriber will adjust it to your body's responses.

What "going up" feels like: usually a few days of mild side effects — a touch of nausea, some fatigue, a dip in appetite — as your body adjusts to the new level. Think of

it as a mini version of week one, each time you increase. It passes. And if it doesn't pass, or it's severe, that's information for your prescriber, not a personal failure. Some women stay at lower doses for their whole journey and do great. Some move up quickly. Your path is yours.

Two honest warnings about titration. First: don't rush it. I know you want the weight off faster, and I know it's tempting to push the dose up early — but the schedule exists for a reason, and the women who push it end up sick, not skinny. Second: don't skip increases either, assuming "less is more." The therapeutic effect — the appetite suppression that actually changes your life — often arrives only at higher doses. The schedule is a balance, and your prescriber is the one holding the scales.

The side effects: the full honest list

Let's run through the common ones, because knowing what's normal is half the battle.

Nausea. The king of GLP-1 side effects. It's caused directly by the slower gastric emptying — food sits in your stomach longer, and your stomach complains. It's most common in the first weeks, worst in the day or two after each injection, and usually worst when your stomach is completely empty or when you've eaten something too rich or too large. Nausea is the side effect we'll spend a whole chapter defeating in the kitchen, and there's a survival kit for it below.

Vomiting. The more aggressive cousin of nausea. For most women it's occasional and tied to specific mistakes — eating too much, eating too greasy, eating too late, or being at a new dose. If it happens once or twice, it's miserable but manageable. If it's frequent or you can't keep fluids down, that's a call-your-doctor situation, and I'll give you the exact red flags in a minute.

Diarrhea. Common, especially early on and especially after dose increases. The practical advice: hydrate aggressively, and eat bland, binding foods (rice, bananas, toast, applesauce — the BRAT classics) until it settles. If it lasts more than a couple of days or comes with fever or pain, call your doctor.

Constipation. The mirror image, and honestly more common than people admit. Slower digestion means slower everything. This one creeps up on women in weeks two and three, and it's worth being proactive: water, fiber from gentle sources, movement, and if needed, a stool softener — check with your doctor first. Nobody talks about this at the dinner party, but everyone on this medication has opinions about it.

Fatigue. The first weeks are genuinely tiring. Your body is adjusting, you're eating less, and that combination is a lot. The fix is mostly patience plus the basics: protein, hydration, sleep, and not fighting the tiredness with caffeine. It usually fades by month two.

Headache. Often dehydration in disguise — especially if you've been nauseous or eating less. Water and electrolytes first. If it's severe or persistent, or accompanied by vision changes, call your doctor, because that's a different conversation.

When it's normal — and when to call the doctor

Here's the triage system that will keep you sane. **Most side effects are normal** if they're uncomfortable but manageable — you're nauseous but you can eat small things, you're tired but you can function, you had a rough day but it's passing. That's the deal you signed up for, and it gets better.

Call your doctor if: you can't keep fluids down for more than a day (dehydration is a real risk and can land you in the ER); you have diarrhea that won't stop; you have severe constipation that's painful or won't resolve; or you have any symptom that feels wrong or scary to you. You are not bothering them. This is literally their job.

And here are the red flags that mean call immediately or go to the ER: severe abdominal pain — especially pain that's sharp, radiates to your back, or comes with nausea and vomiting, because that can signal pancreatitis (inflammation of the pancreas, a known risk with these drugs); a swollen face, lips, or tongue, hives, or trouble breathing — signs of an allergic reaction; or pain in your upper right abdomen that could signal

gallbladder trouble. If in doubt, err on the side of calling. The worst outcome of a phone call is thirty seconds of embarrassment. The worst outcome of ignoring severe abdominal pain is not worth thinking about.

The nausea survival kit

Let's build the kit that got me through month one. Keep these things in your house, your bag, and your car — because nausea doesn't respect your schedule.

Ginger, in every form. Ginger chews, ginger tea, crystallized ginger, ginger ale (flat, if the bubbles bother you). Ginger genuinely helps nausea — it's not a placebo, it's one of the few remedies with actual science behind it. Keep a stash in your purse.

Small, frequent meals. This is not optional on these medications — it's the whole game. Your stomach empties slowly now, so a normal meal is too much, and an empty stomach is worse. Six small things a day beats three big ones, every single time. Crackers and a protein shake at 10 a.m. is not "grazing," it's strategy.

Protein shakes. Your best friend on bad days. They're easy to get down when nothing else appeals, they're gentle on a queasy stomach, and they keep you from the under-eating spiral. Keep a case on hand before you need it. (Full protein strategy coming in the next chapter — for now, just know they're the safety net.)

Electrolyte drinks. When you're nauseous or not eating much, your electrolytes get depleted, which makes the nausea and headache worse — a vicious cycle. Sipping an electrolyte drink (the zero-sugar kind, or a hydration powder you like) breaks the cycle. Room temperature is easier than ice-cold when you're queasy.

Bland basics. Saltines, plain toast, white rice, bananas, applesauce, plain broth. Boring is the point. Save the exciting food for later months; in month one, your stomach wants predictable and plain.

Cold foods. Sometimes the smell of warm food is the trigger. Cold or room-temperature food — a yogurt, a protein shake, some watermelon, a cheese stick — is often easier to face than anything you have to smell cooking.

One weird trick that works: sniffing an isopropyl alcohol wipe can actually calm nausea fast — it's a trick used in emergency rooms, and it's oddly effective. A few sniffs, and the wave passes. Keep a couple in your bag. Nobody will know what you're doing, and you'll feel like you have a secret superpower.

Injection day strategy

The mechanics matter more than you'd think. Here's the system that works.

Where: the injection goes into the fatty tissue — most commonly the abdomen, thigh, or upper arm. The abdomen is the most popular (easiest to reach, and some women find it absorbs slightly faster). Whatever site you choose, **rotate it** — don't hit the same spot every week. Left side of the belly, right side, left thigh, right thigh — a simple rotation prevents soreness and irritation.

When: the same day every week, at a time that suits you. Morning works for some; evening (right before bed) works for others — if you're prone to nausea, sleeping through the first hours can help. Pick your anchor and set the phone reminder.

How: take the pen out of the fridge about 30 minutes before you inject — a cold injection stings more. Wash your hands, clean the site with an alcohol wipe, and inject per the instructions your pharmacy or prescriber gave you. It takes about ten seconds. It barely hurts — most women describe it as a quick pinch, far less painful than they feared.

The day after: plan for it. Keep it low-key, keep the easy food on hand, stay hydrated. By month two you won't think twice about it, but in month one, treat injection day like a recovery day — because being kind to yourself on that day is what gets you through the month.

The most important expectation for week one

I'm going to end where I started, because I want this to land: **it's okay if week one is boring.** It's okay if you feel nothing. It's okay if you don't lose a pound in the first ten days. It's okay if your only symptom is being tired. The medication is a slow-building tool, and the starting dose is a whisper, not a shout.

The women who quit in week one — who decide "it's not working" or "I can't handle this" — are quitting before the story even starts. The women who make it are the ones who understand that month one is tuition: it's the month you pay to learn your body's new rules, so that months two through twelve can be the payoff.

Give it the month. Give it the titration. Give it the protein and the ginger tea and the boring rice. And if you're still miserable at the end of the month — genuinely miserable, not just adjusting — that's information, and your prescriber can work with it. But don't judge the whole journey on day three. The arc is the story, not the first chapter.

Toolkit Summary

- Week one is often uneventful — that's normal; the starting dose is deliberately low and effects build over weeks.

- Titration exists to let your body adapt: don't rush dose increases, and don't skip them — your prescriber's schedule is the balance.
- Common side effects: nausea, vomiting, diarrhea, constipation, fatigue, headache — most are manageable and fade by month two.
- Under-eating is the biggest first-month mistake: it amplifies nausea, fatigue, and misery instead of speeding results.
- Call your doctor for anything that won't stop or that scares you; go to the ER for severe abdominal pain (pancreatitis), allergic reactions, or signs of gallbladder trouble.
- The nausea survival kit: ginger in every form, small frequent meals, protein shakes, electrolytes, bland basics, cold foods — and alcohol wipes to sniff when a wave hits.
- Injection day: rotate sites (abdomen, thigh, upper arm), same day each week, room-temperature pen, and plan a low-key day after.

Chapter 5: The Protein Priority: Eating for Muscle, Not Just Weight Loss

HERE'S THE STATISTIC THAT SHOULD BE PRINTED ON EVERY PRESCRIPTION PAD: WHEN WOMEN LOSE WEIGHT ON GLP-1 MEDICATIONS, A MEANINGFUL PORTION OF WHAT THEY LOSE IS NOT FAT — IT'S MUSCLE. STUDIES SUGGEST THAT A SIGNIFICANT SHARE OF THE WEIGHT LOST ON THESE DRUGS CAN BE LEAN MASS, ESPECIALLY WHEN PEOPLE EAT TOO LITTLE AND DON'T EXERCISE. AND HERE'S WHAT THAT MEANS FOR YOU: IT'S ENTIRELY POSSIBLE TO END THIS JOURNEY LIGHTER, WEAKER, AND MORE TIRED THAN YOU STARTED — UNLESS YOU MAKE PROTEIN THE NUMBER ONE PRIORITY ON YOUR PLATE.

I want to say that again, because it's the single most important nutritional idea in this book: **your goal is not just to lose weight. Your goal is to lose fat while keeping your muscle.** The scale will not tell you the difference. Your mirror won't either, at first. But your energy will. Your hair will. Your metabolism will. And in two years, when you're at your goal weight and you have to maintain it, your muscle will be the thing that decides whether you keep it off or slide back.

This chapter is the why and the how — why protein matters more than anything else right now, how much you actually need, and how to get it down when your appetite has disappeared and nothing sounds good.

Why protein is the number one priority

Let me give you four reasons, and they all matter.

Muscle preservation. Your body is losing weight at a rate it's not used to. In that situation, it will happily break down muscle tissue for energy — and if you're not eating enough protein and not using your muscles, it will do exactly that. Every gram of muscle you keep is a gram that keeps your strength, your shape, and your ability to move through life. Every gram you lose makes you weaker, softer, and — here's the cruel part — easier to regain fat later, because muscle burns calories just by existing.

Satiety. Protein is the most filling macronutrient, full stop. On a medication that's already suppressing your appetite, protein is what makes the fullness feel good instead of hollow. A meal built on protein keeps you satisfied for hours; a meal built on carbs leaves you hungry an hour later, reaching for something else. On these medications, you get fewer meals' worth of appetite — so every meal has to count, and protein is what makes it count.

Hair loss prevention. I'm going to be direct with you, because you deserve to know this before it happens: rapid weight loss of any kind can trigger temporary hair shedding, and GLP-1 weight loss is no exception. It's usually a few months in, it's alarming, and for most women it's temporary. But protein (and overall adequate nutrition) is one of the things that genuinely reduces the severity. Your hair is made of protein. Starve your hair of it, and it will let you know.

Metabolism maintenance. Muscle is metabolically active tissue — it burns calories around the clock, even when you're sitting still. When you lose muscle, your resting metabolism drops, which means you burn fewer calories at the same weight, which means maintaining your new weight gets harder. This is a huge part of why women regain after dieting: every diet that stripped muscle made their maintenance math worse. Protein plus strength training is the counter-move. We'll get to the strength training part later — for now, know that protein is the fuel for all of it.

How much protein do you actually need?

Let's talk numbers, in a way that's actually useful and not overwhelming.

The general guidance for women on weight-loss medication is roughly 1.2 to 1.6 grams of protein per kilogram of body weight per day. Let me translate that into American: if you weigh 200 pounds, that's about 110

to 145 grams of protein a day. If you weigh 170 pounds, it's about 95 to 125 grams. For most women starting this journey, a realistic daily target lands somewhere in the **100 to 130 gram range** — and if that sounds like a lot, you're right. It is a lot. It's more than you've ever eaten on purpose, and it's more than most women on these medications actually get.

Here's the honest math problem: you're now eating maybe half the food you used to eat, and you're supposed to fit the same protein into it. That's why protein can't be an afterthought — it has to be the frame of every meal, the thing you build everything else around. And you will not hit the target by accident. You will hit it by design: protein first, at every single eating moment, every single day.

A few practical notes. If you're doing the math and it feels impossible on your current appetite, start where you are — 60 grams a day is better than 30, and 80 is better than 60. Don't let the perfect target stop you from making progress. And check with your doctor or a registered dietitian about the right target for your body, especially if you have kidney considerations — the numbers above are a starting framework, not medical advice.

The protein-first plate

Here's the structure I want you to use for every meal, from now through maintenance. It's three steps, in order:

Step one: protein. Decide the protein first — chicken, fish, eggs, Greek yogurt, cottage cheese, tofu, shrimp, lean beef, turkey. It's the anchor of the plate. Everything else gets built around it.

Step two: vegetables. Whatever vegetables you can tolerate — and on a sensitive stomach, that might be gentle ones (steamed zucchini, roasted carrots, spinach wilted into eggs) rather than big raw salads. Vegetables bring fiber, volume, and nutrients without many calories, which is exactly what you want when you're eating small amounts.

Step three: the rest. Rice, potatoes, bread, fruit, whatever else you're eating — the carbohydrate part of the meal comes last, in a portion that fits the protein. Not eliminated — never eliminated; carbs are not the enemy and you need them for energy. Just after the protein, in proportion to it.

A visual that helps: imagine your plate as a protein anchor with supporting players, not a classic "meat and sides" where the sides are the point. On these medications, the sides are the supporting cast, and the protein is the star. If you can only eat half your plate, eat the protein half.

The protein ladder: whole foods first

When your appetite is small, every bite has to earn its place — so you want the highest-quality protein you can get, in the fewest bites. Think of it as a ladder:

Top rung — whole foods: chicken breast, turkey, fish (especially salmon, tuna, shrimp), eggs, Greek yogurt, cottage cheese, tofu, edamame, lean beef, pork. These are the gold standard: protein plus micronutrients, in a form your body knows how to use. A palm-sized portion of chicken (about 4 ounces) is roughly 25 to 35 grams of protein. A cup of Greek yogurt is about 20. Two eggs are about 12. Cottage cheese is quietly a superstar — about 25 grams per cup, and it's one of the gentlest foods on a queasy stomach.

Middle rung — protein supplements: shakes, protein powders, protein bars, protein water. These are not cheating — they're tools. A shake with 25 to 30 grams of protein is the difference between hitting your target on a bad day and falling to half of it. We'll talk more about the shake strategy in a minute, but the ladder rule is simple: whole foods when you can, supplements when you can't, and no guilt about either.

Bottom rung — protein-adjacent foods: bone broth, protein-fortified things (protein pasta, protein pancakes), collagen, nut butters. These have some protein and can nudge your numbers up, but they shouldn't be your main strategy — you'd need to eat a lot of them to make a real dent.

The ladder principle: always reach for the highest rung you can manage in that moment, and don't beat yourself up when the answer is the shake.

High-protein foods that are gentle on a sensitive stomach

Here's the list I wish I'd had in month one — proteins that are easy to tolerate when you're queasy, and how to eat them:

- **Greek yogurt** — plain, full-fat or low-fat; add a little honey or berries. Cold, smooth, protein-dense, and easy.
- **Cottage cheese** — mild, soft, and oddly soothing; works with a sprinkle of salt and pepper or a little fruit.
- **Eggs** — scrambled or hard-boiled are gentler than fried; eggs are the original safe food.
- **Rotisserie chicken** — already cooked, easy to pick at, and a lifesaver on low-energy days (more on that in the kitchen chapter).
- **White fish** — cod, tilapia, sole: mild, low-fat, and much easier on the stomach than rich fish.
- **Tofu** — soft or silken, in a gentle soup or simply sautéed; bland in the best way.
- **Protein shakes** — the safety net, and the subject of the next section.
- **Shrimp** — surprisingly gentle, quick to cook, and a solid protein hit.

A note on red meat and rich proteins: some women find fatty cuts and heavy meats harder to digest on these medications, thanks to the slowed digestion. If steak

suddenly doesn't sit right, that's not a moral failing — it's your stomach telling you it wants the gentler stuff for a while. Listen to it.

The protein shake safety net

Here's a day that will happen to you: you wake up, you're queasy, nothing sounds good, the thought of chewing food makes you want to cry, and you still need to function — work, kids, life. This is the day the protein shake saves you.

The strategy is simple: **always have shakes on hand, before you need them.** Keep a case in the pantry. Know which brand you like. The ideal shake for your purposes has 20 to 30 grams of protein, not too much sugar (10 grams or less is a good guideline), and a flavor you can actually drink on a bad day.

On a day when nothing sounds good, the goal isn't a gourmet meal — it's something that keeps you from the under-eating spiral. A shake is liquid protein, which means it's easier to get down than food, gentler on the stomach than food, and it doesn't require chewing, smelling, or decision-making. Sip it slowly, over an hour if you need to. Add some crackers if you can manage them. That's a survival meal, and it's a good meal, because it's protecting your muscle on the worst day of your month.

One more shake tip: on good days, blend them into your routine — a shake as your afternoon snack, or half a shake with breakfast. Consistency beats intensity. The women who hit their protein targets are the ones who made the shake a habit, not an emergency tool.

What happens if you don't eat enough protein

Let me paint you the picture of what goes wrong, because I've watched it happen and I want you to avoid it.

If you spend months eating very little, and very little of it protein, here's what follows: the scale goes down — and you feel triumphant — and then, around month three or four, things get weird. You're tired all the time, even though you're lighter. Your hair starts shedding — clumps in the shower, alarming amounts on the brush. You're losing weight but your shape looks... softer, not leaner, because you're losing muscle along with the fat. Your strength is gone — stairs feel harder, not easier. Your nails are brittle. You feel cold. And when you finally reach your goal weight, you discover that your maintenance calories are pitifully low — because you burned your metabolic engine down along with the fat.

This is the "skinny fat" outcome — lighter on the scale, weaker in the body, and facing a brutal maintenance phase where every single calorie counts. It's the most common way these medications go wrong, and it's almost entirely preventable.

The prevention is boring and it works: eat protein at every eating moment, use the shakes as your safety net, and (when you're ready — we'll get there) add strength training so your body has a reason to keep the muscle. Protein without training preserves what you have. Protein plus training builds more. Either way, protein is non-negotiable.

Here's the reframe I want to leave you with: **the number on the scale is not the score.** The score is how you feel in your body, how much strength you kept, how much energy you have, and whether the weight stays off when the medication's job is done. And the score is decided, more than anything else, by what's on your plate. Protein first. Always.

Toolkit Summary

- Losing weight on GLP-1s can mean losing muscle — a meaningful share of the weight can be lean mass if you under-eat protein. Protecting muscle is the whole game.
- Protein preserves muscle, keeps you satisfied, reduces hair shedding, and protects your metabolism — which is what makes maintenance possible later.

- Realistic target for most women: roughly 100–130 grams of protein per day, or about 1.2–1.6 g per kilogram of body weight; confirm your number with your doctor.
- Build every meal protein-first: protein anchor, then vegetables, then carbs — if you can only eat half the plate, eat the protein half.
- Use the protein ladder: whole foods (chicken, fish, eggs, Greek yogurt, cottage cheese, tofu) first; shakes and bars when you need them; no guilt either way.
- Gentle high-protein foods for a sensitive stomach: Greek yogurt, cottage cheese, eggs, rotisserie chicken, white fish, tofu, shrimp.
- Keep a protein shake safety net stocked at all times — 20–30 grams of protein, low sugar — for the days nothing sounds good.
- Skip the protein and you risk muscle loss, hair thinning, fatigue, a slower metabolism, and a brutal maintenance phase — the "skinny fat" outcome. Protein is how you lose fat and keep the you that matters.

Chapter 6: The Nausea-Proof Kitchen: Food That Works When Nothing Sounds Good

HERE'S A SCENE FROM MY FIRST MONTH THAT I THINK ABOUT MORE THAN I'D LIKE: STANDING IN FRONT OF MY OPEN REFRIGERATOR AT 6 P.M., STOMACH CHURNING, HEAD POUNDING, NOTHING IN THAT ENTIRE APPLIANCE LOOKING EDIBLE, AND FEELING GENUINELY PANICKED ABOUT THE PROSPECT OF DINNER. NOT HUNGRY — THE OPPOSITE. BUT ALSO AWARE THAT IF I DIDN'T EAT SOMETHING, I WAS GOING TO FEEL EVEN WORSE BY 9 P.M., AND WORSE STILL TOMORROW.

That's the paradox of month one on these medications: you have almost no appetite, and yet eating is the single most important thing you can do. The women who struggle are the ones who try to power through on willpower and an empty stomach. The women who thrive are the ones who built a kitchen that works with a queasy, suppressed appetite — a kitchen where the answer to "what can I eat?" is never "nothing."

This chapter is that kitchen. The staples, the systems, the survival menus, and the grocery list that got me through month one — so that when the nausea hits, you already have a plan.

The nausea-proof staples list

These are the foods that got me through the worst weeks. They're bland, they're boring, and that's exactly the point — your stomach wants predictable right now, and these are the most predictable foods in existence:

- **Saltines and plain crackers** — the universal safe food. Keep them on your nightstand, in your bag, in your car. A cracker or two the moment you wake up can head off morning nausea before it starts.
- **Plain white rice** — the gentlest carbohydrate on earth. A small bowl with a little salt is a complete meal when nothing else works.
- **Bananas** — soft, mild, easy, and they bring potassium, which you'll be losing.
- **Applesauce** — smooth, cool, soothing. The unsweetened kind is best.
- **Plain toast or English muffins** — dry, boring, perfect. Don't even think about the butter if you're queasy.
- **Broth** — chicken or vegetable, sipped warm from a mug. It counts as food and hydration, which is a two-for-one on a bad day.
- **Oatmeal** — plain, made with water or milk, eaten warm and slow. Add a little honey if you can handle it.
- **Ginger tea** — the nausea remedy that actually works. Keep a box in the pantry at all times.

- **Scrambled eggs** — soft, protein-rich, and one of the most tolerable foods on the planet. No cheese, no butter, no chives — just eggs.
- **Greek yogurt** — cold, smooth, protein-dense. The plain kind with a drizzle of honey if you want it sweeter.
- **Cottage cheese** — mild, soft, and genuinely soothing. Salt and pepper, or a few berries.
- **Watermelon and other melon** — cold, hydrating, and easy on the stomach.
- **Popsicles** — the real-fruit kind, not the sugar-water kind. They're hydration you can hold in your hand.
- **Electrolyte drinks** — the zero-sugar kind, or a hydration powder. Room temperature is easier than ice-cold when you're queasy.
- **Protein shakes** — your safety net, always stocked, always ready. (The full strategy lives in the protein chapter — this is where you keep them.)

That's the list. It's not exciting. It's not meant to be. It's the difference between eating something and eating nothing on the days when your stomach is running the show.

The "safe foods" concept — and why repetition is fine

Here's something I need to give you permission for, because every diet you've ever been on taught you the opposite: **it is completely fine to eat the same foods over and over for a while.**

Diet culture told you variety was a virtue — different colors on the plate, new recipes every week, a "balanced" rotation. That's great advice for someone with a normal appetite. It's terrible advice for someone whose stomach is queasy and whose appetite is the size of a thimble. On a bad day, the last thing you want is a new recipe with unfamiliar textures and smells. You want the thing you know works. The thing you've eaten six times this week already.

That's what "safe foods" means: the short list of things your body reliably accepts. Not the healthiest things, not the most interesting things — the things that go down and stay down. For some women it's crackers and broth for a week straight. For others it's yogurt and bananas. Whatever your list is, honor it. Your nutrition will average out over months, not days. A week of crackers and protein shakes is not a nutritional failure — it's a survival strategy, and it's the right call.

The one rule that matters: **every safe food should still carry some protein or nutrition when you can manage it.** Boring is fine; empty is not. A week of

crackers alone leaves you weak and miserable. A week of crackers plus yogurt, eggs, and protein shakes keeps your body running while your stomach figures itself out.

Small, frequent meals: the whole game

I'm going to say this in the most direct way I can: **three meals a day will not work for you right now.** Not because you're doing it wrong, but because your stomach empties slowly on this medication — a big meal sits there like a brick, and a completely empty stomach is its own kind of misery. The nausea on these drugs loves two states: too full and too empty.

The fix is grazing, done deliberately. Five or six small eating moments a day instead of three big ones:

- A cracker and a few bites of yogurt when you wake up
- A protein shake mid-morning
- A small bowl of soup or rice at lunch
- A handful of something in the afternoon — a cheese stick, some melon
- A small dinner — protein first, a little rice, maybe some vegetables
- A final small something before bed, if you can manage it — a few bites of cottage cheese, half a banana

Nothing on this schedule is a "meal" by the old definition, and that's the point. Each small eating moment is gentle on your stomach and keeps your blood sugar, your energy, and your nausea in a stable place. You're not snacking mindlessly — you're feeding your body on its new terms.

The 5-bite rule

Here's a practical tool for the days when even a small plate feels like too much. I call it the 5-bite rule, and it's exactly what it sounds like:

Put a small amount of food in front of you — and commit to five bites. That's all. Five bites, then stop and reassess.

Not the whole plate. Not "I should finish this." Five bites. If after five bites you feel okay — not great, just okay — take a few more, slowly. If after five bites you feel worse, you're done, guilt-free, and you can try again in an hour or two.

This rule does three things. It takes the pressure off — the psychological weight of "I have to eat a meal" is often heavier than the nausea itself. It prevents the over-full misery that comes from eating too much too fast. And it builds the habit of checking in with your body mid-meal, which is a skill you'll need for the rest of your life on these medications — knowing the difference between "I could eat more" and "I'm done" before the discomfort arrives.

A note for the food-waste warriors among us: yes, you'll throw away food you would once have finished. That's going to happen, and it's going to feel strange, and it's okay. Leftovers are your friend. Small portions on small plates are your friend. The plate you don't finish is not a moral failure — it's your body setting a new boundary, and you're allowed to listen.

The survival day menu

Let me give you a full day of eating for the worst day of your month — the day you wake up queasy and it never quite lifts. This is the menu that got me through more than one of those days:

- **On waking:** two saltines and a few sips of ginger tea, before you get out of bed if the nausea is bad. Let it settle for ten minutes.
- **Mid-morning:** a protein shake, sipped slowly over an hour. Half a banana if you can manage it.
- **Lunch:** a mug of warm broth and a handful of crackers. Maybe half a cup of plain rice if you're feeling brave.
- **Afternoon:** a few spoonfuls of Greek yogurt or cottage cheese. An electrolyte drink if you haven't had one today — you probably need it.
- **Dinner:** a small portion of plain chicken or scrambled eggs with a little rice. Five bites, then reassess. Stop when your body says stop.

- **Before bed:** a few bites of applesauce or another cracker or two — going to bed with a completely empty stomach is a setup for a rough morning.

This is not a balanced diet. It's a survival day, and survival days are allowed. Notice what's in there, though: even on the worst day, there's protein (the shake, the yogurt, the eggs) and hydration (the broth, the electrolyte drink, the tea). That's the difference between surviving and crashing. Your only job on a survival day is to keep your body fueled and hydrated enough to get to tomorrow — and tomorrow is usually better.

What to avoid (at least for now)

You know what's coming next, but let me be specific, because knowing why makes it easier to comply:

- **Greasy and fried foods.** The slowed digestion means fat sits in your stomach like a bowling ball. French fries, fried chicken, heavy pizza — these will make you miserable in ways you can't imagine until you've felt it.
- **Spicy foods.** Your stomach's tolerance for heat drops dramatically on these medications. What you loved in January will punish you in March.
- **Rich, creamy, heavy foods.** Cream sauces, big cheese plates, butter-soaked anything. Save them for a future you who can handle them.

- **Carbonated drinks.** The bubbles expand in a stomach that's already slow to empty, and the result is bloating and pressure that feels awful. Flat ginger ale is fine — fizzy is not.
- **Big raw salads.** Yes, really — raw vegetables are hard to digest, and on a slowed stomach they can cause bloating and cramping. Cooked or roasted vegetables are much gentler.
- **Alcohol.** Aside from the empty calories, alcohol hits harder on these medications and can trigger nausea. Most women find they simply don't want it anymore — that's the medication talking, and it's fine to listen.
- **Eating late at night.** A full stomach lying down equals reflux and nausea. Try to have your last food a few hours before bed.

Here's the reassurance: this list is not forever. As your body adjusts — often by month two or three — your tolerance expands, and most women can gradually reintroduce foods they love. The avoidance is a season, not a sentence. For now, the goal is to not make a queasy stomach angrier, and these are the foods that do exactly that.

Cooking shortcuts for low-energy days

When you're nauseous and tired, the barrier between you and food is not appetite — it's effort. The answer isn't to summon more willpower; it's to remove the effort. Here are the shortcuts that matter:

- **The rotisserie chicken is your friend.** Already cooked, already seasoned, pick at it cold or warm it up. A rotisserie chicken plus a bag of microwave rice is a real dinner with zero cooking.
- **Frozen is fine.** Frozen vegetables steam in the microwave in four minutes. Frozen rice pouches, frozen chicken strips, frozen berry mixes for yogurt — none of it is cheating, and all of it removes the "I have to cook" barrier.
- **Microwave > stove.** Your standards are on vacation this month. The microwave is a legitimate kitchen appliance and you will use it without apology.
- **Pre-made protein.** Hard-boiled eggs you bought already peeled. Greek yogurt cups. String cheese. Protein shakes. Protein bars. Deli turkey. These require zero assembly and they're the difference between eating and not eating at 3 p.m.
- **Cook once, eat three times.** On a good day, make extra rice, extra chicken, extra eggs — then the bad days are just assembly, not cooking.

- **Paper plates.** This sounds ridiculous, but the dishes pile up fast when you're eating six small things a day, and a sink full of dishes is a barrier you don't need. Use the paper plates. This is a season.

The grocery list that got me through month one

Here's the actual list, so you can build your kitchen before you start — before you need it, while you still feel good:

The pantry: saltines, plain crackers, white rice (or microwave rice pouches), oatmeal, chicken broth and vegetable broth, ginger tea, honey, applesauce cups, canned soup (the mild ones — chicken noodle, tomato), protein shakes (a case — trust me), protein bars, electrolyte drink mix, low-sugar popsicles.

The fridge: Greek yogurt (plain), cottage cheese, eggs, a rotisserie chicken (or two), deli turkey, string cheese, soft tofu, fresh ginger, bananas, watermelon or melon, berries, lemons (for the water).

The freezer: frozen vegetables (peas, spinach, broccoli), frozen rice, frozen chicken strips, frozen berry mix, a backup rotisserie chicken if you're feeling organized.

That's it. It's a boring list, and it's the right list. Every item on it is gentle, quick, or both — which is exactly what your first month demands.

Here's the last thing I want you to know about the nausea-proof kitchen: it's not about loving this season. It's about surviving it well enough to get to the next one — the season where your appetite stabilizes, your safe-food list grows, and eating becomes normal again. The kitchen you build now is the bridge to that season. And when you cross that bridge — and you will — you'll look back on the crackers and the ginger tea and the rotisserie chicken with something close to affection. They got you through.

Toolkit Summary

- Stock the nausea-proof staples before you need them: saltines, white rice, bananas, applesauce, toast, broth, oatmeal, ginger tea, eggs, yogurt, cottage cheese, melon, popsicles, electrolytes, protein shakes.
- "Safe foods" are the short list your body accepts — repetition is fine, boring is fine; just make sure every safe food still carries protein or nutrition when you can manage it.
- Eat five or six small eating moments a day instead of three meals — a too-full stomach and a too-empty stomach are both nausea triggers.

- Use the 5-bite rule: commit to five bites, stop, reassess. It removes the pressure and teaches you to check in mid-meal.
- The survival day menu: crackers and ginger tea on waking, a protein shake mid-morning, broth and crackers at lunch, yogurt in the afternoon, a small protein dinner, a little something before bed — protein and hydration even on the worst day.
- Avoid for now: greasy, fried, spicy, rich, creamy foods, carbonated drinks, big raw salads, alcohol, and late-night eating. It's a season, not a sentence.
- Remove the effort on low-energy days: rotisserie chicken, frozen everything, microwave meals, pre-peeled eggs, paper plates.
- The month-one grocery list is short and boring on purpose — gentle and quick are the only requirements right now.

Chapter 7: Water, Electrolytes, and the Digestion Rules

THE FIRST THING NOBODY TELLS YOU ABOUT GLP-1 MEDICATIONS IS THAT THEY TURN YOUR BODY INTO A NEGOTIATION. YOUR STOMACH NOW EMPTIES SLOWLY. YOUR APPETITE IS A STRANGER WHO SHOWS UP LATE AND LEAVES EARLY. YOUR THIRST — WHICH YOU NEVER THOUGHT ABOUT BEFORE — QUIETLY STOPS SHOWING UP AT ALL. AND YOUR DIGESTIVE SYSTEM, WHICH HAS BEEN RUNNING ON AUTOPILOT FOR THIRTY YEARS, SUDDENLY REQUIRES A USER'S MANUAL.

This chapter is that manual: how to drink enough when your body stops asking, how to keep your electrolytes from going sideways, the eating rules that prevent most of the misery, and what to do when alcohol hits you like a freight train.

None of this is medical advice. It's the collected wisdom of thousands of women who've walked this path — and the lessons I learned the hard way.

Why Water Matters More on These Medications

These medications work by slowing digestion — which makes you feel full, but also means water sits differently, constipation becomes a real risk, and dehydration, already the most common reason people feel terrible on this medication, sneaks up on you faster than you'd think.

Water does more heavy lifting on GLP-1s than on any other weight-loss plan you've tried:

- **It keeps things moving.** The number one side effect of these medications is constipation, and water is the cheapest laxative on the planet. Fiber without water is like sawdust in a dry pipe — it makes things worse.
- **It dulls the nausea.** Dehydration is a nausea amplifier — the single best non-medication fix for low-grade queasiness is sipping water all day.
- **It protects your kidneys.** Your kidneys filter more byproducts during rapid weight loss and need fluid to do it — your bloodwork will thank you.
- **It fights the fatigue.** That weird, heavy tiredness in month one is often dehydration, not the medication — you're eating a fraction of what you used to, and your body is pulling water from everywhere it can.

- **It keeps the scale honest.** Water weight fluctuations are why the scale does its infuriating two-steps-forward-one-step-back dance; being well hydrated keeps that noise to a minimum.

I'll say it plainly: if you do nothing else from this book, drink more water. It fixes more problems than anything else I'll tell you.

How Much, and How to Know You're Getting It

The old "eight glasses a day" rule is a decent start, but on these medications most women need more. A practical target is roughly 64 to 100 ounces a day — about two to three liters — and closer to the higher end if you're nauseated, vomiting, having diarrhea, or training.

Don't chug it — chugging water on a stomach that empties slowly is a fast track to feeling bloated and miserable. Sip all day, keep a bottle in sight (desk, car, couch), and use a straw, which for reasons I can't explain makes drinking feel less like a chore. Add flavor if plain water bores you — lemon, cucumber, a splash of sugar-free electrolyte mix.

The most reliable test is the color of your urine: pale straw is the goal. Dark yellow means you're behind.

The Thirst Signals That Disappear

Here's a weird side effect nobody warned me about: these medications blunt your thirst the same way they blunt your hunger. Your brain simply stops sending the "you should probably drink something" signal. I once went a full day without drinking anything — not avoiding water, just never thinking of it.

That's the trap. You can't rely on thirst to tell you when to drink anymore. You have to drink on a schedule, the way you'd take a medication. If you wait until you feel thirsty, you're already dehydrated.

Other sneaky signs: headaches in the afternoon, darker urine, dry mouth, dizziness when you stand up fast, and a fuzzy head that reads the same sentence three times. Feel any of those? Drink a glass of water before you blame the medication — it often isn't the medication at all.

Electrolytes: The Unsexy Plot Twist

Water gets all the glory, but electrolytes are the quiet hero of this story. Electrolytes are the minerals your body needs to conduct electricity — literally — for your muscles, your nerves, and your heart. The main players: sodium, potassium, and magnesium.

On a normal diet, you get plenty of these from food without thinking. But on GLP-1s, three things go wrong at once: you're eating much less, so fewer minerals are

coming in; your body is dumping water as you lose weight, and electrolytes go with it; and nausea, vomiting, or diarrhea can drain them fast — especially potassium.

The result is a collection of symptoms that feel like side effects of the medication but are actually depletion: muscle cramps (especially at night, in your calves), headaches that won't quit, fatigue, heart palpitations or a racing heart, dizziness, and jitters that make you think you drank too much coffee.

If you experience a racing heart, chest pain, or severe weakness, that's a call to your doctor — not a "drink some Gatorade" moment. But for the everyday low-grade stuff, replenishing electrolytes often clears it up within a day.

Easy Electrolyte Sources

You don't need a lab and a spreadsheet. You need a few reliable tools:

- **Electrolyte powders or tablets.** Look for ones with actual sodium and potassium — not just a vitamin pack with an electrolyte label. A single serving mixed into water, once or twice a day, is the easiest fix there is. On days you're nauseated or have had diarrhea, double up.
- **Broth or bouillon.** A warm cup of broth is sodium, fluid, and comfort in one. On days when food sounds like an insult, broth is the answer.

- **A pinch of salt.** If you're eating a very low-calorie diet, don't be afraid of salt. Your body needs it. It's the processed-food salt flood you're avoiding, not sodium itself.
- **Potassium-rich foods that are gentle on a queasy stomach:** potatoes (even mashed), bananas, avocado, and coconut water.
- **Magnesium.** Many women find a magnesium supplement (especially glycinate or citrate) helps with both cramps and constipation — ask your doctor or pharmacist about dose and form.

One more note: on days when you're vomiting or having diarrhea, plain water isn't enough — you need the electrolytes too. That's the difference between recovering in hours and feeling wrecked for days.

The Digestion Rules

Because these medications slow your stomach's emptying, digestion becomes a conscious act. The good news: there's a set of simple rules that prevents most of the nausea, the reflux, and the "why do I feel like a snake that ate a goat" sensation. Follow them like they're the law for the first few months, and your stomach will stop negotiating.

Rule one: eat slowly. And I don't mean "chew a little more." I mean deliberately, painfully slowly — put your fork down between bites, take a sip of water, breathe. A meal that used to take you ten minutes should now take

twenty-five. Your stomach is processing at a fraction of its old speed, and if you eat at your old pace, you'll be full before your brain gets the message — then miserable for hours after.

Rule two: chew thoroughly. The more you chew, the less work your stomach has to do, and the easier everything passes through. Aim for chewing until the food is basically mush. It feels absurd at first. It works anyway.

Rule three: stop at 80 percent full. The food keeps coming after you stop eating — you can feel fine at the end of a meal, then twenty minutes later feel like you've eaten Thanksgiving plus the turkey. So stop when you're no longer hungry. Do not wait for "full." Full is where the nausea lives.

Rule four: no lying down after eating. This one is non-negotiable. With your stomach emptying slowly, lying down after a meal is an open invitation for reflux, heartburn, and — in the worst cases — food and stomach acid coming back up while you're asleep, which can be genuinely dangerous. Give yourself two to three hours upright after eating. If you need a nap after a meal, nap in a recliner or propped up with pillows.

Rule five: small meals, more often. Six small meals beats three big ones every single time on these medications — you'll hit your protein and calories without ever overloading your stomach.

Constipation: The Most Common Complaint

If there's one side effect that unites every woman on this medication, it's constipation. It's the most common reason people quit — not because it's unbearable, but because it's relentless and nobody prepared them for it. The cause is simple: slower digestion, less food, and often less water equals things grinding to a halt.

Your first line of defense is prevention, and it's the same trio as always: water, movement, and fiber. But here's the catch — if you're already stopped up, adding a sudden flood of fiber will make it worse, because fiber needs water to work and your pipes are already dry. So: water first, always. Then fiber, added gradually — chia seeds, oats, berries, vegetables, and a fiber supplement like psyllium husk if you need it. Then movement — a daily walk genuinely helps get things moving.

Magnesium deserves its own mention. Many women find magnesium citrate or glycinate at night handles both the constipation and the leg cramps, gently — but that's a conversation for your doctor or pharmacist, not me.

On laxatives: over-the-counter stimulant laxatives are fine occasionally for a genuine emergency, but they're not a lifestyle. If you're reaching for them weekly, talk to your doctor — there are prescription options that work with the medication's slowed motility instead of against it. You do not have to just live with this. Ask.

If constipation is accompanied by severe abdominal pain, vomiting, or you haven't gone in over a week, call your doctor — don't white-knuckle it.

When Diarrhea Happens

The flip side: sometimes the pendulum swings the other way, especially when you change doses or eat something rich. Diarrhea on these medications is usually short-lived, but it's rough when it hits, because it drains fluid and electrolytes fast.

If it happens: hydrate aggressively with electrolyte drinks, not just water. Eat small, bland, gentle things — toast, rice, bananas, applesauce, broth — and skip anything fatty or spicy. Skip dairy and alcohol for a day or two. And don't panic: one bad day is not a trend. Call your doctor if it lasts more than a couple of days, if you see blood, if you can't keep fluids down, or if you're dizzy and weak — that's dehydration territory, and it deserves real attention.

Alcohol: It Hits Different Now

This is the warning I want to tattoo on your arm: alcohol on GLP-1s is not the same drug you knew. It hits harder, it lasts longer, and it will humble you in public.

Why? Your stomach empties slowly, so alcohol absorbs unpredictably. You're eating less, so there's less food to buffer it. And the medication itself seems to change how

alcohol affects many people — studies suggest GLP-1s reduce the desire to drink, and plenty of women report going from nightly wine to "I don't really want it." The result: you may want alcohol less, and the alcohol you do drink will hit you like you're twenty-one and untrained.

Practical rules that will save you:

- **Assume your tolerance is a third of what it was.** One glass of wine may now feel like two.
- **Never drink on an empty stomach.** On this medication, that's a one-way ticket to nausea, dizziness, and a very bad night.
- **Sip slowly and alternate with water.** One drink, then a full glass of water.
- **Skip the sugary cocktails.** A margarita on a stomach that empties slowly is a sugar bomb with delayed delivery. The nausea isn't worth it.
- **Know your medications.** If you're also on a sulfonylurea or insulin, alcohol can drop your blood sugar dangerously, and the usual "eat something" fix may not work the same way with slowed digestion. Have this conversation with your doctor.
- **If you're nauseated or vomiting, don't drink at all.** Your body is already fighting; don't hand it a second opponent.

And if you find the desire for alcohol has simply evaporated — welcome to one of the stranger side effects. For many women, the quieting of food noise extends to wine o'clock. Some find it disorienting; others, liberating. Either way, it's normal.

Caffeine: Proceed With Curiosity

Coffee and GLP-1s have a complicated relationship. For many women, coffee becomes a nausea trigger they never had before — the morning cup that used to be sacred now sits wrong. For others, caffeine jitters get amplified because there's less food to absorb it and you're running on less energy overall.

The good news: caffeine isn't off-limits — the rules are just more careful. Don't drink it on a completely empty stomach; eat a little something first. If you're jittery, anxious, or your heart is racing, dial it back. And since caffeine is a mild diuretic, match your coffee with water, glass for glass.

And decaf exists — it's not a punishment. On the days your stomach is angry, a warm decaf with a splash of cream scratches the ritual itch without the gamble.

Putting It All Together

Here's what a well-managed day on these medications looks like: wake up, glass of water. Small breakfast with protein, eaten slowly. Water bottle refilled at your desk,

sipped all morning. A walk after lunch, upright and moving. An afternoon electrolyte drink if you've been struggling. A modest dinner, stopped at 80 percent full, followed by two hours of not lying down. Magnesium at night if you need it. Bed.

It's not glamorous or complicated — just a set of small rules that keep your body from fighting the medication, and keep you feeling human while it works.

Toolkit Summary

- Drink 64–100 ounces of water daily — sip constantly, don't chug, and use the color of your urine as your guide.
- Thirst signals fade on GLP-1s, so drink on a schedule, not on demand.
- Electrolytes matter: watch for cramps, headaches, fatigue, and dizziness; keep electrolyte powder, broth, and potassium-rich foods on hand.
- Eat slowly, chew thoroughly, and stop at 80 percent full — "full" is where nausea lives.
- Stay upright for two to three hours after eating to prevent reflux.
- Prevent constipation with water, gradual fiber, movement, and magnesium; call your doctor if it's severe or paired with pain.

- Alcohol tolerance drops dramatically — drink less, slower, with food, and never when nauseated.
- Caffeine is fine with caution: not on an empty stomach, and matched with extra water.

Chapter 8: Strength Training on GLP-1s: Protecting Your Muscle

I'M GOING TO TELL YOU SOMETHING THAT MIGHT MAKE YOU WANT TO CLOSE THIS BOOK, AND I NEED YOU TO STAY WITH ME: THE SCALE IS LYING TO YOU. NOT MALICIOUSLY — IT'S JUST INCOMPLETE. WHEN YOU LOSE WEIGHT ON THESE MEDICATIONS, A MEANINGFUL CHUNK OF WHAT YOU LOSE IS NOT FAT. IT'S MUSCLE. AND MUSCLE IS THE ONE THING YOU CANNOT AFFORD TO LOSE, BECAUSE IT'S THE THING THAT DECIDES WHETHER YOU KEEP THE WEIGHT OFF FOR GOOD.

The research is sobering. In the clinical trials, people lost significant weight — but a substantial portion of that weight, in some cases as much as 20 to 40 percent, was lean mass. That's muscle, bone, and other tissue you actually need. It's the difference between losing weight and losing the right weight.

Here's the good news: this is the most fixable problem in the entire book. You do not need a gym membership, a personal trainer, or a single piece of equipment you don't already own. You need a basic plan, two to three sessions a week, and the willingness to be a beginner at something again.

Let me tell you the story of how I learned this the hard way, so you don't have to.

Why Muscle Matters More Than the Scale

Muscle is not just about how you look in a tank top, although that's a lovely side effect. Muscle is your body's engine. It burns calories around the clock — even when you're sitting on the couch, even when you're asleep. Every pound of muscle you carry raises your resting metabolic rate, which means the more muscle you keep, the more food your body burns just by existing.

And that's the whole game. Here's the math nobody explains: when you lose weight, your body burns fewer calories because there's less of you. That's why diets plateau. But if you lose fat and keep muscle, your metabolism stays stronger — which means when you transition to maintenance, you can eat more without regaining. Muscle is literally the difference between keeping the weight off and playing whack-a-mole with the scale for the rest of your life.

Muscle also gives you the shape women actually want when they say they want to lose weight. The toned arm, the lifted backside, the defined waist — that's not weight loss. That's muscle under a thinner layer of fat. Two women can weigh exactly the same and look completely different, and the difference is always the same word: muscle.

And then there's the long game. Muscle protects your bones, your joints, and your balance as you age. Building muscle in your forties and fifties is one of the most

powerful things you can do for your seventy-year-old self. You're not just losing weight right now — you're building the body you'll live in for the rest of your life.

The Muscle Loss Problem

Here's what happens without training: the body, confronted with rapid weight loss, doesn't discriminate. It burns fat and muscle. When you lose weight very quickly — which GLP-1s are designed to do — the muscle loss can be significant. Studies of rapid weight loss consistently show that without resistance training and adequate protein, a large share of what's lost is lean tissue. That's not a flaw in the medication. That's biology. The medication takes the weight off; you decide what kind of weight it is.

The consequences of losing that muscle are sneaky and cruel. First, your metabolism drops, which makes the plateau phase hit harder and earlier. Second, your shape changes in ways you won't love — the "skinny fat" look, where you're smaller but soft, and everything feels a little deflated. Third, and most importantly for the long game, the muscle loss makes regain almost inevitable, because your body is now burning fewer calories than it should for your size.

The fix is so simple it sounds like a scam: lift things. That's it. Resistance training tells your body, "I need this muscle, keep it." Combined with eating enough protein, it

flips the script — instead of losing muscle, you lose fat and hold onto (or even build) the tissue that keeps your metabolism humming.

The Strength Training Basics for Beginners

If you've never lifted a weight in your life, I need you to hear this: you are the perfect candidate for this plan. Beginners get the fastest results of anyone. Your body has never been asked to do this before, so it responds like a plant that's finally been watered.

How often: Two to three sessions a week, on non-consecutive days. That's it. You do not need to live in a gym. You need forty-five minutes, three times a week, forever. The "forever" part matters more than the intensity. Consistency beats heroics every single time.

What to do: Compound movements — exercises that work multiple muscle groups at once. These give you the most return on your time investment. Your basic menu:

- **Squats** (or sit-to-stand from a chair, if you're starting from zero)
- **Hip hinges** — deadlifts, hip thrusts, or glute bridges
- **Push** — push-ups (on knees or against a wall is fine), chest presses, shoulder presses
- **Pull** — rows, lat pulldowns, or resistance band pulls

- **Carry** — farmer's carries with dumbbells or heavy grocery bags

That's a full-body workout. Pick one exercise from each category, do three sets of eight to twelve reps, rest a minute between sets, and you're done. Twenty minutes to half an hour, twice or three times a week.

Progressive overload, explained like you're me: This is the fancy term that makes strength training sound complicated, and it's actually dead simple. It means: do a little more than last time. Each week, either add a tiny bit of weight, add one more rep, or add one more set. That's the entire secret. Your muscles only grow when you give them a reason to, and "a little more than last time" is the reason. You don't need a spreadsheet — though you'll want to jot down what you lifted, because you will absolutely forget.

Form over everything: In the beginning, light weight and correct form beat heavy weight and ugly form. Record yourself, watch a few videos of the movement, or — if you can swing it — book two or three sessions with a trainer to learn the basics. Think of it as paying for a lesson in how to use your own body.

Bodyweight vs. Gym: The Honest Comparison

Let me settle this debate for you: both work, and the best one is the one you'll actually do.

Bodyweight training — squats, lunges, push-ups, glute bridges, planks — is free, portable, and completely sufficient for the first several months. It's how I started, in my living room, in athletic shorts that had seen better days. The catch is that bodyweight exercises get easy, and once they get easy, you need to make them harder — which means you either get creative (slower reps, one-legged squats, more reps) or you eventually need some weight.

The gym gives you the easiest path to progressive overload, because you can add five pounds at a time forever. A pair of adjustable dumbbells at home gives you about 80 percent of the gym's benefit for a fraction of the hassle. Resistance bands are another great cheap option.

My honest advice: start wherever you are, right now, with zero shame about it. If that's your living room, your living room is a gym. When you outgrow it — and you will, because you're going to get stronger — then you upgrade. The worst thing you can do is wait until you feel "ready" for a gym. Ready never comes.

Should You Train on Injection Day?

This is one of the most common questions in every support group, and the answer is: there is no rule, only data about your own body.

Some women train on injection day with zero issues. Others find that the day after injection — when the medication peaks in their system — brings fatigue, queasiness, or just a general "please don't make me do things" feeling. Neither is wrong.

Practical guidance: if you're new, keep your training schedule simple and consistent, and let your energy decide which days you train. If you find injection day leaves you drained, train the day before or two days after. If you feel fine, train whenever. The one thing that doesn't work is waiting to "feel like it" — motivation is unreliable, so anchor your training to days, not feelings.

If you're tired, train anyway — but train lighter. A short, easy session still counts. The habit is the point. Show up, do something, go home. That's a win.

Protein Timing Around Training

I'm going to save you from an entire internet rabbit hole: the timing of protein around your workout matters much less than the total amount you eat all day. The magic window you've read about is real, but it's a door, not a gate — it's open for hours, not minutes.

The simple version: get your daily protein (for most women on these medications, roughly 80 to 120 grams a day, or about 0.7 to 1 gram per pound of body weight — your doctor or a protein calculator can give you your number), and make sure some of it lands near your workout. A shake or a high-protein snack within an hour

or two of training is perfect. It doesn't have to be fancy. A shake after your session is the easiest habit there is, because it also doubles as hydration and a recovery treat.

The real timing rule that matters: don't train on empty if you can avoid it, and don't skip protein afterward because you "aren't hungry." On these medications, appetite is a liar. Your muscles need the protein whether or not your brain is asking for it.

What If You've Never Lifted Weights?

First: welcome. You're in excellent company, and you're about to discover something wonderful about your body.

If you're starting from absolute zero, here's your first week: do a gentle full-body session twice, using just your body weight or the lightest possible weights. Squat to a chair and stand back up. Push against a wall. Pull a resistance band. Carry two full water bottles across the room a few times. That's it. That's a workout. It counts.

The most common mistake beginners make is doing too much, too hard, too soon, and then quitting when they're sore for four days. Your first month, the goal is not to feel destroyed — the goal is to build the habit and learn the movements. Soreness should be mild and brief. If you can't sit on the toilet without groaning, you overdid it, and that's okay — it happens to everyone — but dial it back next time.

A few beginner-level resources that will genuinely help: the free apps like Nike Training Club and FitOn have guided beginner strength programs that tell you exactly what to do and show you the moves. YouTube is full of quality form tutorials. And if your budget allows, two or three sessions with a personal trainer — specifically to learn squat, hinge, push, and pull form — is the best money you'll spend on this entire journey.

One more thing: you will be bad at this at first. That is the point. Every strong woman you admire was once a beginner doing shaky squats in her living room. The only way to skip being bad at it is to not do it at all, and that's the one path I won't let you take.

Walking: The Cardio Complement

Here's a truth that will save you hours of treadmill misery: you don't need cardio to lose weight on these medications. The medication handles the calorie deficit. What you need cardio for is everything else — your heart, your lungs, your mood, your digestion, and your sanity.

Walking is the perfect complement to strength training. It's free, it's gentle, it's good for your joints, and it helps with two of the most common GLP-1 side effects: constipation (movement gets things moving) and nausea (fresh air and gentle motion genuinely help). Walking after meals also helps steady your blood sugar.

Aim for a daily walk — even twenty minutes counts — and if you can, add a slightly longer one on weekends. Don't overthink pace. Don't wear a watch that judges you. Just walk. The goal is habit, not performance. Strength training builds the engine; walking keeps it tuned.

The Big Picture

Here's the reframe I want to leave you with: you are not on a weight-loss journey. You're on a body-composition journey. The medication will handle the fat. Your job — the one thing the medication cannot do for you — is to protect and build the muscle that keeps the weight off, shapes your body, and carries you through the rest of your life.

Two to three sessions a week. Compound movements. A little more than last time. Protein all day. A daily walk. That's the whole program. It's not complicated — it's just consistent.

And when you step on the scale six months from now and see a number that doesn't look as low as you hoped, remember this chapter. Muscle weighs more than fat. If you're lifting, your scale is not telling you the whole story — and the story it's hiding is the best one.

Toolkit Summary

- Without training, 20–40 percent of weight lost on GLP-1s can be muscle — protect it with resistance training.
- Muscle keeps your metabolism high, shapes your body, and is the key to keeping weight off long-term.
- Train two to three times a week with compound movements: squat, hinge, push, pull, carry.
- Progressive overload simply means doing a little more than last time — more reps, more weight, or more sets.
- Bodyweight training works; the gym just makes progression easier. Start where you are, today.
- There's no rule about injection-day training — anchor your sessions to days, not energy levels.
- Total daily protein matters most; a shake near your workout is a simple, effective habit.
- Walking is the ideal cardio complement — daily, gentle, and good for digestion and mood.
- Beginners get the fastest results. Be a beginner on purpose; it's a temporary state.

Chapter 9: The Body-Fat Recomposition Mindset: Numbers, Scales, and Patience

LET ME TELL YOU ABOUT THE MORNING I NEARLY QUIT. I WAS FOUR MONTHS IN, DOING EVERYTHING RIGHT — PROTEIN, WATER, STRENGTH TRAINING, SLEEP. THE SCALE HAD NOT MOVED IN ELEVEN DAYS. ELEVEN DAYS OF THE SAME NUMBER, STARING UP AT ME LIKE A DARE. I STOOD THERE IN THE BATHROOM, HOLDING THE SCALE, GENUINELY CONSIDERING THROWING IT OUT THE WINDOW, AND I THOUGHT: WHAT IS EVEN THE POINT?

Here's what I know now that I didn't know then: those eleven days were not a failure. They were my body doing something the scale can't see — changing shape, recompensing, holding water while muscle was being built. The scale was lying, and I was about to quit over a lie.

This chapter is about the psychological side of this journey, which is the side nobody prepares you for. The shot is easy. The eating is manageable. The real battle — the one that decides whether you stay the course — is in

your head. It's about the scale, the plateaus, the identity shift, and the uncomfortable question of who you are when the food noise finally goes quiet.

The Scale Obsession

Here's the fundamental problem with the scale: it measures everything you are, all at once, and reports it as one number. Water. Food in your system. Glycogen. Hormones. Muscle. Fat. Bone. The contents of your bladder. All of it, smushed into a single digit that your brain treats as a verdict on your worth.

On GLP-1s, this gets amplified, because the weight comes off so fast in the beginning. That early rapid loss is intoxicating — you step on the scale daily, sometimes twice, watching the numbers fall like a slot machine paying out. And then it slows down, as it must, and the same scale that made you feel invincible now makes you feel like you're failing.

The truth about daily weighing: your weight legitimately fluctuates two to five pounds a day, just from water, salt, hormones, and the simple mechanics of being alive. On these medications, add in delayed digestion — the food sitting in your stomach — and the daily number is almost meaningless. Weighing daily is like checking the stock market every hour: pure noise, maximum anxiety.

Weekly vs. Monthly Weigh-Ins

My advice, hard-won: weigh in once a week, on the same day, at the same time, under the same conditions — first thing in the morning, after the bathroom, before food or water. Put the scale away the rest of the week. Out of sight, out of the spiral.

If you're someone who genuinely can't handle the scale — if even weekly weigh-ins hijack your whole day — switch to monthly. Or ditch the scale entirely and use measurements and photos instead. I know women who lost a hundred pounds and never weighed themselves once, and their results are indistinguishable from the scale-trackers. The scale is a tool, not a god. You get to choose which tools serve you.

One more rule that will save you: when you weigh in, don't react. Just record the number and move on with your day. No celebrating, no mourning — because both are overreactions to a snapshot that will be different in twenty-four hours. Let the trends do the talking, not the days.

The Plateau Problem: Why It Happens

Every single person on this medication hits a plateau, and almost everyone panics. So let me explain what's actually happening, because it's rarely what you think.

Reason one: water weight games. When you lose fat, your fat cells don't just disappear — they empty out and temporarily fill with water before shrinking. This is the famous "whoosh" effect: the scale stalls for a week or two, then suddenly drops several pounds overnight. The stall wasn't a stall. Your body was doing the paperwork.

Reason two: your body adapted. As you lose weight, your body burns fewer calories because there's less of you. This is normal, expected physiology — not a personal failing. It's why weight loss always slows over time, on any plan, for every human who has ever lived.

Reason three: you're losing inches, not pounds. If you're strength training, you may be losing fat and gaining muscle simultaneously. The scale doesn't move. The mirror does. This is the single most common cause of "plateaus" in women who are doing everything right — and it's the best kind of plateau there is.

Reason four: you're under-eating. This one is counterintuitive, but it's real. Eat too little for too long and your body downshifts — it burns fewer calories, holds onto water, and generally digs in its heels. Starving yourself to break a plateau is like pressing the gas pedal while the parking brake is on.

What to do about a plateau: don't panic, don't starve, don't add more medication on your own. Reassess the basics — protein, water, steps, sleep — and fix anything that's slipped. Trust the process for two to three weeks. If the scale truly hasn't moved in a month and your measurements haven't either, that's a conversation with

your doctor about your dose and your calorie target. Plateaus end. Every single one. The question is whether you quit before yours does.

Body Recomposition: When the Scale Lies

I want to introduce you to the concept that saved my sanity: body recomposition. It means changing your body's composition — less fat, more muscle — without necessarily changing your weight. It's possible because muscle is denser than fat. A pound of muscle is smaller than a pound of fat, the way a pound of lead is smaller than a pound of feathers.

So you can lose five pounds of fat, gain five pounds of muscle, weigh exactly the same, and look dramatically different. Smaller waist. Firmer arms. Clothes that fit differently. And the scale? The scale says nothing changed. The scale is a liar.

This is why measurements and photos matter more than the number. A measuring tape tells the truth the scale won't: track your waist, hips, thighs, and upper arms once a month. Take photos once a month — same spot, same lighting, same clothes. When you're convinced you're stalled, lay the photos side by side. The photos don't lie. The photos show you what three months of invisible progress actually looks like.

And here's the beautiful part: recomposition means you don't have to choose between losing weight and building strength. You get to do both. You get to lift weights, eat your protein, and trust that the changes are happening even when the scale sits stubbornly still.

Measurements and Photos Over Numbers

Let me be specific about this system, because it changed everything for me. Once a month — same day, same time — I take three measurements (waist at the narrowest point, hips at the widest, one thigh), I take two photos (front and side, same spot in my hallway, same black leggings), and I weigh myself once. The rest of the month, I don't weigh at all.

The measurements and photos are the source of truth. The scale is just a data point. And when I have a month where the scale doesn't move but my waist is down an inch, I have proof — physical, visual proof — that I'm not failing. I'm recomposing. The scale was never going to tell me that.

Also: keep your old clothes. Not to shame yourself — to measure yourself. That pair of jeans from before is the most honest progress tracker you own. The day they zip easily is worth more than any number on a scale.

Who Am I Without Food Noise?

This is the question nobody warns you about, and it's the deepest psychological challenge of this journey. For many of us, food was never just food. It was the reward at the end of a hard day. It was the comfort when we were sad. It was the entertainment when we were bored. It was the social glue, the celebration, the thing we thought about constantly — the background hum of our inner lives.

Then the medication turns the hum off. The food noise goes quiet. And what's left is a silence that can feel, at first, like an empty room.

You might find yourself standing in the kitchen, not hungry, with nothing to do. You might realize you don't know what to do with your hands, your evenings, your feelings. You might feel a strange grief for the relationship with food you thought you hated — because it was also a relationship, and it filled space in your life.

This is normal. This is the part they don't put in the commercials. And it's the part that matters most, because if you don't fill that space with something, the food noise will come back — and when it does, it will come back louder.

The work here is identity work. Who are you when you're not the person who's always thinking about food? What do you actually enjoy? What do you do with a

Tuesday evening that isn't ordered around a meal? What did food give you — comfort, control, company — and how do you get those things from somewhere else?

Start small. Reconnect with hobbies you abandoned. Take walks without a destination. Call the friend you used to eat with, and just talk instead. Sit with the uncomfortable feelings instead of eating them — not because you're strong, but because the medication has given you the rare gift of feeling them clearly for the first time.

The Emotional Void Food Used to Fill

Let's be honest about what food did for us. It was the coping mechanism that was always available, always legal, always there. Bad day at work? Food. Argument with your partner? Food. Bored on a Sunday? Food. Celebration? Food. Sadness? Food. It worked — that's why we used it.

When the medication removes the appetite, it removes the coping mechanism, and all those feelings that food used to absorb are suddenly right there, unabsorbed, demanding attention.

This is why so many women on these medications report feeling emotionally raw in the first few months. It's not depression from the drug (though that deserves a conversation with your doctor if it's persistent). It's the sudden absence of the emotional anesthetic you didn't know you were using.

What works: finding replacement coping mechanisms that actually work. Movement, for one — a walk genuinely changes a bad mood. Journaling, for another — getting the feeling out of your head and onto paper. Talking to someone, whether that's a friend, a partner, or a therapist. And therapy deserves a real mention here: if food has been your primary coping mechanism for years, a therapist who specializes in eating behaviors can be the difference between temporary weight loss and permanent change. This is not a weakness. This is maintenance for your brain, and your brain is doing the hardest job on this journey.

Body Dysmorphia on Rapid Weight Loss

Here's a cruel twist nobody prepares you for: the faster you lose weight, the longer it takes your brain to catch up. You can lose forty pounds and still see the person from forty pounds ago in the mirror. You can buy new clothes and still feel like you're wearing a costume. You can have people telling you how great you look and feel like they're talking about someone else.

This is called body dysmorphia, and it's startlingly common on rapid weight loss. Your brain built its self-image over decades, and it doesn't update on the medication's timeline. The mirror shows you a stranger, and your brain fills in the old familiar image instead.

The practical fixes: keep the monthly photos — they're your ground truth when your brain lies to you. Let your clothes be the evidence. And be patient with the lag. For many women, it takes a year or more — a year of living in the new body — before the mirror and the brain finally agree.

If the dysmorphia is severe — if you can't leave the house, if you're constantly distressed about your appearance, if you're developing disordered behaviors around food — please talk to a professional. This is a real thing, it happens to real people, and it's treatable. You don't have to white-knuckle your way through a distorted self-image.

The Patience Reality

Here's the number I want you to internalize: half a pound to two pounds a week. That's it. That's healthy, sustainable, keep-it-off-for-good weight loss. It's not dramatic. It doesn't make for good commercials. But it's the rate that actually sticks, because it's the rate that gives your body time to adjust, your skin time to adapt, and your brain time to catch up with the mirror.

The early weeks of GLP-1 treatment are a beautiful lie — the weight falls off fast, and your brain concludes this is how it will always be. It won't. The loss will slow to the sane, sustainable rate, and then it will plateau, and then it

will resume, and then it will plateau again. This is not a malfunction. This is the shape of every successful weight-loss journey in human history: stair steps, not a slide.

So here's your new definition of success: not a number on a scale by a certain date, but the slow, boring, miraculous process of your body changing — month after month, photo after photo, measurement after measurement — while you keep showing up.

The women who succeed on this medication are not the ones who lost the fastest. They're the ones who refused to quit during the boring parts. The scale will move. The photos will change. The plateau will end. Your only job is to still be there when it does.

Toolkit Summary

- The scale measures everything at once and lies about it — weigh weekly or monthly, under the same conditions, and don't react to single days.
- Plateaus are normal and temporary: water shifts, metabolic adaptation, recomposition, and under-eating all masquerade as stalls.
- Body recomposition means losing fat while gaining muscle — the scale stays still while your body changes shape.
- Track measurements and monthly photos; they tell the truth the scale won't.

- When food noise goes quiet, an emotional void appears — fill it with movement, hobbies, connection, and therapy if needed.
- Rapid weight loss can cause body dysmorphia; your brain lags behind your body, and photos are your ground truth.
- Healthy loss is 0.5-2 pounds per week — slow, boring, and sustainable. The tortoise keeps the weight off.
- Plateaus end. Every single one. Your job is to still be there when yours does.

Chapter 10: Staying on Track: Holidays, Plateaus, and Bad Days

THERE'S A MOMENT IN EVERY WEIGHT-LOSS JOURNEY WHEN THE NOVELTY WEARS OFF AND REALITY SETS IN. THE HONEYMOON — THE THRILLING FIRST MONTHS WHEN THE POUNDS MELT OFF AND EVERY DAY FEELS LIKE A WIN — IS OVER. THE SIDE EFFECTS HAVE SETTLED, THE WEIGHT LOSS HAS SLOWED TO A SANE, BORING CRAWL, AND YOU REALIZE YOU'RE NOT ON A SPRINT. YOU'RE ON A LONG, SLOW HAUL THROUGH THE REST OF YOUR LIFE, WHICH INCONVENIENTLY INCLUDES THANKSGIVING, VACATIONS, BIRTHDAYS, AND THE OCCASIONAL DAY WHERE YOU EAT AN ENTIRE SLEEVE OF COOKIES AND FEEL LIKE A FRAUD.

This chapter is the long-haul playbook. It's about the real-life events that derail people — the holidays, the trips, the plateaus, the bad days, the well-meaning relatives — and how to walk through them without walking off the cliff.

Holidays and Celebrations: The Game Plan

Here's the philosophy I want you to adopt, and I need you to really hear it: on the day you love, eat the thing you love. Not a sad little version of it. The real thing. A

real piece of your grandmother's pie, eaten slowly, with joy, at the actual holiday table, surrounded by actual people.

This sounds like permission to fall off the wagon. It's the opposite. It's permission to stop treating food like the enemy and start treating it like a normal part of a real life. The reason this works — the reason the "eat the thing" philosophy beats the "I'll be good today" approach — is that deprivation is a powder keg. When you tell yourself you can't have the pie, the pie becomes the most powerful food in the universe. You'll obsess over it, resent everyone eating it, and eventually eat three slices of it on a random Tuesday when you're alone and sad. One slice at Thanksgiving, eaten without guilt, is a rounding error in your journey. Three slices on a random Tuesday is a spiral.

The rest of the game plan is logistics:

- **Don't arrive starving.** Eat your normal protein-rich meals earlier in the day. Showing up ravenous is how you end up eating the cheese ball before anyone sits down.
- **Protein first, always.** At a holiday meal, put the protein on your plate first — turkey, ham, whatever's there. Then the vegetables. Then, if you want it, the pie. This ordering genuinely changes how full you feel and how much you eat.

- **Use the 80 percent rule from your everyday toolkit.** Stop before full. The food will still be there tomorrow; your stomach's tolerance won't be.
- **One plate, no rematches.** Give yourself one honest plate and then move away from the food table. It's much easier to not go back than it is to stop once you're there.
- **Alcohol rules apply double on holidays.** Your tolerance is a third of what it was, and the whole family is watching. Sip, space it with water, and know when to stop.
- **Plan the walk.** A family walk after the big meal isn't just good for digestion — it's good for you psychologically, and it models the habit for everyone around you.

And the most important rule of all: one day of eating differently does not undo months of progress. It literally cannot. You would have to eat thousands of calories above maintenance for weeks to gain back real weight. One Thanksgiving is a blip, not a betrayal. Enjoy the blip.

Vacations on GLP-1s

Vacations are where the logistical stuff gets real, and the good news is it's all manageable with a little planning.

The medication itself. First, know your pen's rules: most GLP-1 pens are fine at room temperature for a few weeks (typically around 21 to 30 days after first use — check your specific medication's package insert), which means most trips don't require refrigeration at all. If you're traveling longer or your medication requires cold storage, a small insulated bag with an ice pack does the job. Keep it in your carry-on, not your checked luggage — both because of temperature and because checked bags get lost.

Flying with injections. This is completely allowed. Medications and injection supplies are permitted in carry-on bags, and you can bring them even if they exceed the usual liquid limits — just tell the TSA agent you have medication, and declare it. If you're nervous, bring your prescription label or a note from your doctor. I've flown with pens, needles, and ice packs more times than I can count, and the script is always the same: "I have medication that needs to stay cold," and the agent waves you through.

Timing across time zones. If you're traveling across time zones, the simplest approach is to keep taking your injection on your usual schedule, adjusted to your new local time — if you inject Sunday morning at home, inject Sunday morning wherever you are. If you're traveling on injection day, it's totally fine to shift it by a day. Missing a dose by a day or two is not a crisis; just don't double up.

Eating on vacation. You will likely find you eat less than everyone else, and that's fine. Don't force yourself to eat at the all-inclusive buffet just because you paid for it. Order the protein, skip the bread basket, eat slowly. And the biggest vacation trap is dehydration — flying, sun, and alcohol conspire against you. Water bottle, always. Electrolytes if you're in the heat. Your vacation self will thank you.

One more thing: vacations are for living. If you have a day where you eat the local specialty and drink a margarita by the pool, that's not a failure — that's a life, and the whole point of this journey is to have one.

Plateau Busters: What Actually Works

We covered the psychology of plateaus before, but let's get tactical, because when you're in one, you want actions, not reassurance. Here's what actually works, in order:

Protein. Up it. Most women on these medications under-eat protein, and protein is the single most powerful lever for both muscle preservation and satiety. If you're stalled, look at your protein first. Can you hit your target every single day, no exceptions?

Water. Every stall I've ever had was preceded by a stretch of mediocre hydration. Re-commit to the water game for a week and watch what happens.

Steps. Movement that isn't formal exercise — walking, shopping, puttering — is the "nonexercise activity" that quietly burns a lot of calories. If you're stalled, add steps. Not workouts. Steps. A 20 percent increase in daily walking is often enough to nudge things.

Sleep. Poor sleep raises the hormones that make you hungry and lowers the ones that make you full, and it makes your body hold water. If you're sleeping badly, no amount of diet discipline will fully compensate. Fix the sleep first.

And absolutely not starving. This is the counterintuitive one, and it's the one everyone gets wrong. When you're stalled, the instinct is to cut calories further. Resist it. Eating too little tells your body to conserve, and it will respond by burning less and holding on to everything. If anything, a few days of eating at maintenance — your target calories plus a couple hundred — can "reset" a stubborn plateau by reminding your body that food is available and it can let go. This isn't a hack; it's biology, and it works.

What doesn't work: adding more medication on your own, skipping meals, extreme fasting, and punishing cardio. The plateau-busting answer is boring. It's the basics, done better, with patience.

The One Bad Day Recovery

Let's talk about the cookie incident. You will have a bad day. Maybe a whole bad weekend. You'll eat things you didn't plan to eat, skip your walk, skip your water, and feel like you've undone everything. Here's what I need you to know: you haven't. You can't undo months in a weekend. The only thing that can undo your progress is quitting, and quitting is a decision you make on a Tuesday, not a slide you fall into on a Sunday.

The one bad day protocol, which I've tested extensively:

- **No shame spiral.** The shame is not a motivator. It's the thing that turns one bad day into a bad month, because shame tells you you've already failed, so why bother? You haven't failed. You've had a day. Everyone has days.
- **No punishing.** Do not fast the next day to "make up for it." Do not do double workouts. Punishment reinforces the idea that food is a crime and you are a criminal, which is exactly the mindset you're trying to leave behind.
- **Get back on schedule at the next meal.** Not Monday. Not after vacation. The very next meal. Eat your protein, drink your water, take your walk. The fastest way out of a bad day is a normal day, and a normal day starts with the next meal.

- **Figure out what happened, without judgment.** Did you skip protein and get ravenous? Did you drink too much? Were you exhausted? The bad day is data, not a verdict. One sentence of honest reflection beats three hours of self-flagellation.

Here's the deeper truth: the women who keep the weight off are not the ones who never have bad days. They're the ones who have bad days and simply resume. Recovery is the skill. The bad day is guaranteed; the recovery is the choice.

Managing Social Pressure

You will lose weight, and people will have opinions. This is one of the most predictable parts of the entire journey, and it still catches everyone off guard.

First, the "you've lost enough" chorus. It will come from people who love you and people who barely know you. Sometimes it's genuine concern; sometimes it's projection — your weight loss makes them uncomfortable about their own. Sometimes it's just that people aren't used to seeing you change, and change makes people nervous.

Second, the "you look tired" comments. This one is cruel and common, and it usually translates to "your face looks different," which people can't say, so they say tired instead. It's not about you. It's about their discomfort with a changing face.

Third, the food policing. "Are you going to eat that?" "Just a bite won't hurt." "You're no fun anymore." People will have a lot to say about your plate.

Here's your script, and I want you to practice it in the mirror:

- For concern: "Thanks for caring. My doctor and I are monitoring this closely, and I feel great."
- For comments you don't want: "I appreciate the thought, but I'd rather not talk about my body." Then smile, and change the subject. You are allowed to end this conversation. You are not required to be a captive audience for commentary on your own body.
- For food pushing: "I'm good, thank you." Repeat as needed. No explanations required. "No" is a complete sentence, and you don't need to justify your plate to anyone.

The most important thing to remember: other people's opinions are not your data. Their comfort is not your responsibility. You are doing something hard and brave, and the people who matter will cheer for you, not critique you.

The Mental Game of Month Four Through Eight

If you're in months four through eight, you're in the hardest stretch of the whole journey, and almost nobody talks about it. Here's why it's hard: the novelty is gone, the dramatic early losses are over, the side effects have faded, and the weight loss has settled into a slow grind. The compliments have slowed down. The honeymoon is over, and you're just... living with it. Injecting weekly. Eating protein. Lifting weights. Watching the scale creep down a pound at a time.

This is where most people quit. Not because anything is wrong, but because nothing is exciting anymore. The dopamine of the early months is gone, and what's left is the actual work.

The mental strategies that get you through:

- **Switch the metric.** If the scale is boring you, switch to performance goals — add five pounds to your squat, walk a new route, lift a heavier weight. Progress that isn't weight keeps the momentum alive.
- **Find your people.** The support communities — online and in person — are lifelines in this stretch. Women who are six months ahead of you can tell you what's coming, and women who are six months behind remind you how far you've come.

- **Stop waiting to be motivated.** Motivation is a feeling, and feelings are unreliable. Consistency is a system. You don't need to feel like lifting today; you need to show up because it's Tuesday and Tuesdays are lift days.
- **Celebrate the boring.** A pound this month is a pound you get to keep. Progress that feels slow is still progress that compounds. The women who succeed here are the ones who learned to love the slow months, because the slow months are where the habit is built.
- **Plan for the future.** This is the stretch where the maintenance question starts to feel real. Start thinking about what happens after — the habits you're building now are the ones that will carry you through the rest of your life.

The bad news: months four through eight are hard. The good news: they're hard for everyone, which means they're survivable, which means you're not broken when you struggle. The women who make it through this stretch are the women who keep showing up on the boring days. That's the whole secret. That's always been the whole secret.

Toolkit Summary

- On holidays, eat the thing you love — one real slice, without guilt — and manage the rest with protein-first ordering and the 80 percent rule.

- Travel is manageable: most pens survive trips at room temperature, fly with medication in your carry-on, and adjust injection timing by a day without panic.
- Plateau busters that work: protein, water, steps, sleep — and never starving, which backfires.
- One bad day can't undo months of progress; recover at the very next meal, with no shame and no punishment.
- Social pressure is managed with a script and a boundary: "I'd rather not talk about my body" ends the conversation.
- Months four through eight are the hardest stretch for everyone — consistency, not motivation, is what carries you through.
- The slow months are where the habit is built. The women who succeed are the ones who keep showing up on the boring days.

Chapter 11: The End Game: Maintenance, Tapering, and Life After the Injection

IF YOU ONLY READ ONE CHAPTER OF THIS BOOK, LET IT BE THIS ONE. NOT BECAUSE THE REST DOESN'T MATTER — IT DOES — BUT BECAUSE THIS IS THE PART ALMOST NOBODY PREPARES YOU FOR, AND IT'S THE PART THAT DECIDES WHETHER ALL YOUR HARD WORK BECOMES A PERMANENT CHANGE OR A CHAPTER OF YOUR LIFE YOU'LL TELL STORIES ABOUT LATER.

Here's the uncomfortable truth at the center of it: these medications are not cures. They're treatments. They work while they're in your system, and when they leave, the physiology that made you struggle with your weight in the first place comes back. The clinical trials tell the story plainly: in the studies where people stopped the medication, a large share of the lost weight came back. In the extension studies — the ones where people stopped taking the drug after a year — participants regained most of what they'd lost within a year of stopping.

This is not a failure of the medication, and it's not a failure of you. It's biology. Your body defends its weight like a castle defends its walls, and when you remove the medication that was lowering the drawbridge, the

defenses return. The question this chapter answers is not "will this happen?" — it's "what are we going to do about it?"

And the answer is: a lot. More than you think. You are not doomed to regain. You are walking into the most important phase of this journey, armed with something the clinical trials can't measure — the habits, the skills, and the knowledge you've built. Let me show you how to use them.

The GLP-1 Cliff

The "GLP-1 cliff" is what the support groups call the moment the medication stops and the weight starts coming back. It's called a cliff because it feels sudden — you stop injecting, your appetite returns with a vengeance, and the food noise that went quiet for a year comes back like an old friend who never actually left.

Let me be honest about what the statistics show. In the extension phase of one major study of a GLP-1 medication, people who stopped the drug after a year of treatment regained roughly two-thirds of the weight they'd lost over the following year — while those who stayed on the drug kept it off. Similar patterns appeared across the major trials. The takeaway is stark: for most people, the medication is doing real, ongoing work, and stopping it cold means the work stops too.

But here's what the statistics can't tell you, and what the doom-scrolling headlines always leave out: the studies measured what happened to people who stopped and made no other changes. They measured the cliff. They didn't measure what happens to people who taper slowly, keep their protein habit, keep their training habit, keep weighing in, and treat the transition as the active project it is. You are not a statistic. You're a person with agency, and agency changes the outcome.

So let's talk about the real strategies, starting with the most important conversation you'll have on this entire journey.

The Off-Ramp Conversation With Your Doctor

The decision about when and how to stop — or whether to stop at all — is a medical decision, made with your doctor, based on your body, your labs, your history, and your goals. Not based on what your insurance covers, not based on what your cousin did, and not based on how you feel about needles.

Here's the conversation I want you to have, and I want you to bring questions to it:

- **"What's my target?"** Weight loss medication is usually prescribed with a target range in mind — a weight or a health marker like blood pressure or A1C. Where are you in that range, and what does your doctor consider "done"?
- **"What does the taper look like?"** If you're going to come off, how does your doctor want to do it — drop a dose step at a time, extend the injection interval, or stop outright? Ask for the specific plan, in writing if that helps you.
- **"What's my maintenance dose option?"** The maintenance dose concept is increasingly common: staying on a lower dose indefinitely, like a blood pressure medication, to keep the weight off. This is a legitimate, evidence-supported approach, and your doctor can talk through whether it's right for you.
- **"What do I do if the weight comes back?"** Have the plan before you need it. What's the threshold that triggers a call? What's the threshold for restarting? Knowing the plan in advance takes the panic out of the process.

And here's the honest piece: some doctors are great at starting these medications and less great at the ending. If your doctor doesn't have a thoughtful answer to these

questions, that's not a reason to give up — it's a reason to ask for a referral to an obesity medicine specialist, who deals with this phase every day.

Tapering vs. Cold Turkey

Let me be direct: cold turkey is the worst way to leave these medications, and it's the way most people end up doing it, usually because of insurance or cost or a doctor who says "just stop."

The problem with stopping abruptly isn't just the appetite — it's the shock to your system. Going from a full dose to zero overnight means the food noise doesn't return gradually; it returns like a flood. Your stomach also suddenly empties at full speed again, so you can eat more, more easily, and feel hungry again sooner. It's the perfect storm, and it's why so many people regain quickly after stopping.

Tapering softens all of that. Dropping down a dose step, or extending the interval between injections — say, from weekly to every ten days to every two weeks — gives your body time to adapt to its own appetite signals again. It gives you time to practice the skills you'll need: feeling hungry, eating normally, and stopping at 80 percent full without the medication doing the work for you. Tapering is practice for real life, and real life is what's waiting on the other side.

Whatever your doctor recommends, treat the taper as its own phase — not "the beginning of the end" but "the training wheels coming off, one at a time." You want to arrive at your first fully unmedicated day having already spent months learning to ride without them.

The Maintenance Dose Concept

Here's a reframe that might change how you think about the whole thing: what if stopping isn't the goal?

The maintenance dose concept treats obesity like the chronic condition the medical establishment increasingly understands it to be — something like high blood pressure or diabetes, where you don't "finish" the treatment; you maintain it. In this model, you lose the weight on a therapeutic dose, then drop to a lower maintenance dose that you stay on long-term. The extension studies back this up: people who continued on a lower dose kept off significantly more weight than those who stopped entirely.

The maintenance dose approach isn't for everyone. It has a cost, whether that's out-of-pocket or ongoing side effects, and some people genuinely don't want to be on a medication forever. That's a valid choice. But if your doctor offers it, or if you're dreading the thought of stopping, know that staying on a low dose long-term is not a personal failure or a moral compromise — it's a legitimate medical strategy, no different from taking thyroid medication for a thyroid that doesn't work.

The conversation with your doctor is where this gets decided. Your job is just to know the option exists.

Keeping the Habits When the Medication Leaves

Here's the thing you've been building this whole time without fully realizing it: a set of habits that work with or without the medication. When the injection stops, the habits are what's left. And they are enough — if you keep them.

The protein habit. Your daily protein target was a chore on the medication; now it's a survival skill. Protein is the most satiating nutrient there is, and it's the one that keeps your muscle — and your metabolism — intact. When your appetite returns, protein-first ordering at every meal is the single most powerful thing you can do. You've been training for this for a year. Don't stop now.

The training habit. The strength training you built is your metabolic insurance policy. Muscle burns calories around the clock, and it's the difference between regaining and maintaining. When the medication leaves, the gym doesn't. Two to three sessions a week, forever, no negotiation.

The weighing habit. Weekly weigh-ins felt like a chore; now they're your early warning system. The research on weight maintenance is unanimous: people who keep weighing themselves keep the weight off. The

scale is no longer a judge — it's a smoke detector. You weigh in, you see a trend, you adjust early, when it's easy. That's the whole maintenance game.

The movement habit. The walks, the steps, the movement — it all counts, and it all compounds. Keep moving.

And here's a reframe for the food noise: when it comes back — and for most people, some of it comes back — it's not a personal failing, and it's not a sign you're weak. It's your biology reasserting itself. The difference now is that you have tools it never had before: you know what hunger actually feels like versus craving, you know how to eat slowly, you know what 80 percent full feels like, and you know that one craving doesn't have to become a binge. The skills don't disappear when the medication does. That's the point of having built them.

The Realistic Maintenance Framework

Let's get concrete about what maintenance actually looks like, because "keep the weight off" is too vague to be useful.

Know your set point range. Your body defends a weight range, and that range is partly genetic — it's not a moral judgment, it's your biology's default setting. After a major loss, your body will fight to return toward that range. Knowing that the fight is coming — and that it's

biological, not a failure of will — takes the shame out of the struggle and puts the focus where it belongs: on the tools.

The 5-10 pound buffer rule. This is the single most useful maintenance concept I know. Decide on a buffer: five pounds is a common starting point. When your weight creeps up five pounds above your maintenance target, that's your action trigger — not a crisis, just a signal. For a week, you tighten up: more protein, more water, more steps, earlier nights. Usually that's enough to bring it back down. Ten pounds is the hard line — if you're ten pounds up, that's the moment to call your doctor and have the conversation you planned, before it becomes twenty. The buffer rule works because it converts vague anxiety into clear action: act at five, escalate at ten. You never have to wonder "is this a problem?" again. The numbers tell you.

Weigh-ins as guardrails, not verdicts. Weekly weigh-ins during maintenance aren't about celebrating losses — they're about catching the trend early, when the correction is a week of discipline instead of a full restart. The smoke detector model. Check it, note it, act if needed, move on.

When to consider staying on a low dose. If your history is one of repeated regain — lose, regain, lose, regain — that's not a character flaw, it's a pattern with a name: your body is a strong defender, and it will outlast any willpower-based approach. For people with that

history, a low maintenance dose is often the difference between a lifetime of yo-yo cycling and a stable weight. It's worth a real conversation with your doctor.

You Did This, and You Get to Keep It

And now, the part that doesn't fit in any clinical trial.

You've done something genuinely hard. You took a medication that scared you, you pushed through side effects, you ate more protein than you ever thought possible, you lifted weights when you'd never lifted anything heavier than a suitcase, you survived the plateaus and the bad days and the relatives with opinions. You didn't just lose weight. You built a body, a set of skills, and a relationship with food that you didn't have before.

The medication was a tool. The work was yours.

Here's the identity shift that matters most: you are no longer the person who "tries to lose weight." You are the person who did it — and who knows exactly what it takes to keep it. That's not arrogance; it's evidence. You have the receipts. Every weigh-in, every workout, every protein-heavy meal, every bad day you walked back from — that's all proof of who you are now. And proof is hard to argue with, even for your own inner critic.

The end of the injection is not the end of the story. It's the beginning of the chapter where you find out what you're made of without the training wheels — and you already know the answer, because you've been building it this whole time.

So here's my final instruction, from one woman to another: when the medication leaves, keep the habits. Keep the protein. Keep the training. Keep the weigh-ins. Keep the walks. Use the buffer. Have the conversations. And when the food noise returns, don't panic — greet it like an old acquaintance, use your tools, and let it pass. You did this. You get to keep it. And you will.

Toolkit Summary

- Stopping GLP-1s often leads to significant regain within a year — the "GLP-1 cliff" — but the trials measured people who made no other changes; your habits change the outcome.
- Have the off-ramp conversation with your doctor: target range, taper plan, maintenance dose option, and a pre-planned response to regain.
- Taper slowly rather than stopping cold — it softens the appetite return and gives you time to practice unmedicated skills.
- A low maintenance dose long-term is a legitimate, evidence-supported option, not a moral compromise.

- Keep the habits when the medication leaves: protein-first eating, strength training, weekly weigh-ins, and daily movement.
- Food noise returning is biology, not failure — you now have skills it never had before.
- Use the 5-10 pound buffer rule: act at five pounds up, escalate at ten.
- The medication was a tool; the work was yours. You did this, and you get to keep it.

Resources, Tools, and Further Reading

THIS CHAPTER IS THE APPENDIX YOU'LL ACTUALLY USE — THE REPUTABLE SOURCES, THE CALCULATORS, THE APPS, THE COMMUNITIES, AND THE PRACTICAL EXPLANATIONS OF THE CONFUSING STUFF (LOOKING AT YOU, COMPOUNDING). BOOKMARK IT, DOG-EAR IT, SCREENSHOT THE EMERGENCY SECTION. THIS IS THE STUFF THAT MAKES THE JOURNEY LESS LONELY AND LESS CONFUSING.

Before anything else, the disclaimer that belongs on every page of this book: I'm not a doctor, and nothing in this book — including this chapter — is medical advice. These resources are starting points for conversations with your own healthcare team, not substitutes for them. When in doubt, ask your doctor. When it's an emergency, call 911.

Reputable Sources for the Real Information

The internet will sell you a lot of nonsense about these medications. These are the sources worth your time, because they're run by people whose job is accuracy, not clicks:

- **The Obesity Medicine Association (obesitymedicine.org).** The professional organization for doctors who specialize in obesity treatment. Their patient resources are excellent, and their provider directory is the single best way to find a doctor who actually understands these medications.
- **The CDC's Healthy Weight resources (cdc.gov/healthyweight).** Solid, boring, evidence-based information on weight management, nutrition, and physical activity. Boring is good here. Boring means reliable.
- **The National Institute of Diabetes and Digestive and Kidney Diseases (niddk.nih.gov).** Part of the National Institutes of Health, with clear, plain-language explainers on weight management and the science of obesity.
- **The FDA's medication pages.** When you want the official prescribing information, side effect lists, and safety communications for your specific

medication, go straight to the source. Search the FDA site for your drug name plus "prescribing information."

The Clinical Trial Data, Summarized So You Can Quote It

When people argue with you about whether these medications work, these are the numbers. The trials have made the rounds in headlines, so here's what they actually showed:

- **The STEP trials** studied semaglutide (marketed as Wegovy for weight loss). In the STEP 1 trial, people taking the medication lost about 15 percent of their body weight over 68 weeks — roughly 34 pounds on average — compared to about 2.4 percent for placebo. The STEP 4 trial was the withdrawal study: after a year of treatment, people who were switched to placebo started regaining weight, while those who stayed on the medication kept it off.
- **The SURMOUNT trials** studied tirzepatide (marketed as Zepbound for weight loss). In SURMOUNT-1, people lost up to about 21 percent of their body weight over 72 weeks — more than 50 pounds on average at the highest dose. The SURMOUNT-4 extension study showed the same pattern as STEP 4: people who stopped

the medication regained a large share of the weight — roughly two-thirds within a year — while those who continued maintained their loss.

- **The SELECT trial** studied semaglutide in people with obesity and cardiovascular disease, and found a significant reduction in major cardiovascular events — heart attacks, strokes, and cardiovascular deaths — independent of the weight loss itself.

The takeaway to remember when someone tells you "it's just a diet drug": these medications are being studied for cardiovascular protection, and the regain-after-stopping pattern isn't a scandal — it's the difference between a treatment you're on and a treatment you're not. That's why the maintenance conversation matters so much.

Protein Calculators and Targets

You've heard me say it all book: protein is the priority. Here's how to find your number.

The simple formula most practitioners use: about 0.7 to 1 gram of protein per pound of your goal body weight (or roughly 1.6 to 2.2 grams per kilogram). For most women on these medications, that lands between 80 and 120 grams a day. If you want a calculator rather than doing the math on a napkin, search for "protein calculator" and

use one that asks for your weight and activity level — and take its number to your doctor to confirm what's right for you.

Then make it real: a protein shake or smoothie (20–30 grams), a serving of Greek yogurt (15–20 grams), chicken, fish, eggs, tofu, beans, cottage cheese. Spread it across the day. If you're struggling to hit the target, a protein supplement is a tool, not a failure — think of it as the difference between a meal and a multivitamin.

Free Apps That Actually Help

You don't need to spend money on this journey. These are the free tools that get recommended over and over in the support groups, because they work:

Workout apps for beginners: - **Nike Training Club** — free guided workouts of every level, including beginner strength programs with video demos. Genuinely excellent, genuinely free. - **FitOn** — free workout classes including strength, with instructors who show proper form. - **YouTube** — the world's largest free gym. Search "beginner strength training for women" or "dumbbell workout at home" and you'll find thousands of free, high-quality sessions. The trick is finding a channel you like and sticking with it.

Meal tracking apps: - **MyFitnessPal** — the most popular food tracker, with a huge food database. The free version is enough for most people. - **Lose It!** — a user-friendly tracker with a barcode scanner and clean

interface. - **Cronometer** — the favorite of the protein-and-micronutrient crowd, because it tracks vitamins and minerals in detail. Great if you want to see your electrolytes and magnesium at a glance.

A note on tracking: track if it helps you, and drop it the moment it stops helping. Some women find tracking empowering; others find it obsessive. The goal is awareness, not punishment. If tracking makes you miserable, you have my personal permission to stop — use the protein-first plate method instead.

The Support Communities

You are not the only woman going through this, and the communities where you'll find your people are free and thriving:

- **r/Ozempic, r/Mounjaro, r/Zepbound, r/GLP1**
— Reddit communities where thousands of women share the real, unvarnished experience: side effects, wins, plateaus, injection-day strategies, insurance battles, and everything in between. Search any question you have and you'll find someone who's been through it.

A few ground rules for the communities, from someone who lives in them: they are not medical advice, and anyone who tells you otherwise is a red flag. People share what worked for them, and what worked for them may not work for you. Be skeptical of anyone selling something, promising miracle results, or giving dosing

advice. And when you're deep in the sauce of a bad week, the communities are there for you — but for medical decisions, the doctor wins.

The FDA Compounding Situation, Explained Simply

If you've been confused by the compounding pharmacy news, you're not alone — it's genuinely confusing, and it's changed over time. Here's the simple version.

During the medication shortages a few years back, the FDA allowed licensed compounding pharmacies to make copies of the brand-name medications, so that patients who couldn't get the real thing had an option. When a drug is in shortage, that's legal; when the shortage ends, the FDA's position is that compounding copycats of that drug generally have to wind down.

As the shortages were declared resolved, the FDA began enforcing those restrictions — which is why you saw headlines about compounding pharmacies being told to stop making copycat semaglutide and tirzepatide. There have been court challenges and ongoing debates, and the details have shifted over time. What you need to know:

- There is no FDA-approved "generic" version of these medications. If you see "generic Ozempic," that's marketing, not medicine.

- Legitimate compounded medication requires a prescription from your own doctor and comes from a licensed pharmacy. It should be prescribed based on your specific needs, not ordered from a website like a pair of shoes.
- The FDA has warned about unregulated online sellers of compounded semaglutide, including products using salt forms of the drug that aren't the same as what's in the brand-name pens, and about dosing errors that have sent people to the hospital.
- If you're considering compounded medication, the questions to ask are: Is my doctor prescribing this for me? Is the pharmacy licensed? Am I comfortable that this isn't the same regulated product as the brand-name version?

And the honest bottom line: if you can get the brand-name medication through your insurance or the manufacturer programs below, that's the safest path. If compounding is your only option, go through your doctor and a licensed pharmacy, and keep your BS detector on high.

Insurance Assistance and Manufacturer Programs

The cost of these medications is real, and it's the reason so many people are on compounding in the first place. The manufacturer programs can make a meaningful difference:

- **Novo Nordisk** (makers of Ozempic and Wegovy) offers a savings card for eligible patients with commercial insurance, which can reduce out-of-pocket costs significantly. They also have a patient assistance program for qualifying uninsured patients. Check their website for current eligibility — programs change, so verify before you count on them.
- **Eli Lilly** (makers of Mounjaro and Zepbound) offers similar savings cards for eligible patients with commercial insurance, and has its own patient assistance program through Lilly Cares.

The general rules: savings cards typically require commercial (not government) insurance, and they're not available to people on Medicare or Medicaid. Eligibility changes, so always check the current details on the manufacturer's site. And if your insurance denies coverage, don't just accept it — appeal. The support communities are full of women who won their appeals, and your doctor's office can often help with the paperwork.

Finding an Obesity Medicine Specialist

If your regular doctor isn't giving you the depth you need — or if you're at the maintenance crossroads and want someone who deals with this every day — an obesity medicine specialist is worth the search. These are doctors (and sometimes nurse practitioners) who've done extra training specifically in obesity treatment, and they're light-years ahead of the average clinic on the nuances of these medications.

Two directories to use: the **Obesity Medicine Association's provider directory** (obesitymedicine.org) and the **American Board of Obesity Medicine's "find a physician" tool** (abom.org). Both let you search by location. Many offer telehealth, which widens your options considerably.

What to look for in an appointment: someone who listens, who takes your history seriously, who talks about muscle and protein and maintenance — not just the prescription — and who has a real answer when you ask about the long-term plan. You deserve a doctor who treats this like the chronic condition it is, not a quick fix.

Red Flags: When to Call, When to Go

Most of this journey is manageable at home. Some things are not. Here's your guide to when a symptom is a "call your doctor tomorrow" situation versus a "go to the emergency room now" situation.

Call your doctor promptly for: - Severe or persistent abdominal pain — especially pain in the upper left or center of the belly that radiates to your back (this can signal pancreatitis, a known risk of these medications). - Vomiting that lasts more than a day or prevents you from keeping fluids down — dehydration is the most common cause of hospitalization on these medications. - No bowel movement for a week or more, especially with pain or vomiting. - A lump in your neck or trouble swallowing (a rare thyroid concern that the FDA has flagged). - Signs of an allergic reaction that are mild — skin rash, hives. - New or worsening depression, anxiety, or thoughts of harming yourself. These deserve urgent, compassionate attention — if you're having thoughts of self-harm, call or text **988** (the Suicide and Crisis Lifeline) or go to the emergency room. This is not something to white-knuckle.

Go to the emergency room for: - Severe abdominal pain that doesn't let up. - Inability to keep down any fluids, with signs of dehydration — extreme thirst, dark urine, dizziness when standing, a racing heart. - Swelling of your face, lips, tongue, or throat, or trouble breathing — this is anaphylaxis, and it's an emergency. - Chest pain, shortness of breath, or fainting. - Any symptom that feels truly wrong, even if it's not on this list. You know your body. Trust it.

And one more rule, the one that covers all the others: when in doubt, call. Your doctor's office has a nurse line for a reason. A two-minute phone call that turns out to be nothing is a victory, not a waste.

A Final Word

You've reached the end of the book, which means you've done something most people never do: you've actually prepared. You know about protein and muscle and water. You know what to do on a bad day and a holiday. You know the maintenance questions to ask and the red flags to watch for. You know where to find your people and where to find real information.

That preparation is the difference between hoping it works and knowing what you're doing. You're not hoping anymore. You're ready.

And one last time, because it matters: this book is a companion, not a prescription. Take what serves you, run everything through your own doctor, and trust the work you're doing. You've got this.

Toolkit Summary

- Trust the reputable sources: the Obesity Medicine Association, CDC Healthy Weight, NIDDK, and FDA prescribing information.
- The STEP and SURMOUNT trials show real, significant weight loss — and real regain after stopping, which is why maintenance planning matters.
- Protein target: roughly 0.7–1 gram per pound of goal weight (often 80–120 grams/day for women).

- Free tools that work: Nike Training Club, FitOn, YouTube, MyFitnessPal, Lose It!, Cronometer.
- Support communities (r/Ozempic, r/Mounjaro, r/Zepbound, r/GLP1) are invaluable — but they're not medical advice.
- Compounded medications became legal during shortages and are being restricted as shortages end; use licensed pharmacies, prescriptions, and caution.
- Manufacturer savings cards and patient assistance programs (Novo Nordisk, Eli Lilly) can cut costs significantly — check current eligibility.
- Find an obesity medicine specialist through the OMA or ABOM directories.
- Know your red flags: severe abdominal pain, persistent vomiting, and dehydration warrant urgent care; swelling or trouble breathing warrant the ER.
- This is not medical advice — your doctor is the final authority on your care.

About the Author

Hannah Wells is a health writer who spent two years on GLP-1 medication herself — losing weight, gaining muscle, and documenting every side effect, plateau, and kitchen experiment along the way.

Her work focuses on translating medical research and real-world experience into practical guidance for the millions of women starting these medications without a roadmap. She is not a doctor, and this book is not medical advice — it is the companion she wishes she had on day one.

Hannah lives in Texas with her family, her dog, and a very well-stocked protein shelf.

Also by Hannah Wells

This is her first book.

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